

# Example Peer Recovery Services, Inc.

## Policy and Procedure Manual

Organized to the CARF Behavioral Health Standards Manual, 2026 edition

**DRAFT - the CARF section map behind this document has NOT been checked against the purchased manual. Set manual\_verified: true before use.**

Program: Community Integration  
Effective date: [FILL IN: effective date]  
Approved by: Board of Directors  
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## How to use this manual

This manual is organized by CARF standard area so that a surveyor asking about an area can be handed the matching tab. Each policy names its owner, its review frequency, and -- most importantly -- **the evidence a surveyor will ask for**. A policy with no evidence behind it is the single most common reason an organization does not conform.

**Anything in [SQUARE BRACKETS] or on a fill-in line must be completed by the agency before this manual is adopted.**

See BLANKS\_TO\_COMPLETE.md for the full list.

**Before adoption:** read every policy. These are written to be defensible and to fit a peer-delivered community program, but they are your agency's statements about your agency. Change anything that is not true of you, and change your practice where the policy describes what you intend to do.

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| Area                                               | Policy # | Title                                                | Owner                                       | Review   |
|----------------------------------------------------|----------|------------------------------------------------------|---------------------------------------------|----------|
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## Section 1.A — Leadership

**What this area is about:** Leadership is identified, accountable, and ethical. There is a written code of ethics, a way to raise concerns without retaliation, a corporate compliance program, and evidence that leadership considers cultural competency and the needs of the people served when making decisions.

**Questions a surveyor asks in this area:**

- Show me your org chart and who is accountable for what.
- Show me the signed code of ethics for every staff member.
- Tell me about a time someone raised an ethical concern and what happened.
- How does leadership know what the community you serve needs?

**Who gets interviewed:** CEO, Governing body, Program Director

### 1A-01 — Leadership Structure and Accountability

| Policy # | Owner                   | Review   | Standard area  |
|----------|-------------------------|----------|----------------|
| 1A-01    | Chief Executive Officer | Annually | 1.A Leadership |

#### Purpose

To define who leads Example Peer Recovery Services, Inc., what each leader is accountable for, and how leadership decisions are made and recorded.

#### Policy

Example Peer Recovery Services, Inc. maintains a written leadership structure. Every leadership position has a current written job description stating its authority, its responsibilities, and to whom it reports. Leadership is accountable for the quality of services, the stewardship of resources, the rights of the persons served, and conformance to accreditation, licensure, and contract requirements. No leadership function is left unassigned; where the organization is too small to have a separate person in a role, the role is assigned in writing to a named position.

#### Procedure

1. The Chief Executive Officer maintains a current organizational chart showing every position and reporting line. The chart is dated and re-issued whenever the structure changes.
2. Leadership positions at Example Peer Recovery Services, Inc. are: Chief Executive Officer, Program Director, Clinical/Peer Supervisor, Quality Improvement Coordinator, Safety Officer, Privacy Officer, and Compliance Officer. One person may hold more than one of these roles; the assignment is documented in that person's job description.
3. Leadership meets no less than monthly. Minutes record who attended, what was decided, who is responsible, and by when. Minutes are retained for a minimum of three years.
4. Leadership reviews at each meeting: performance data, incidents, grievances, staffing, and progress on the improvement plan.
5. The Chief Executive Officer reports to Board of Directors on organizational performance no less than quarterly.
6. This policy and the organizational chart are reviewed annually by the Chief Executive Officer and the review is documented.

#### Evidence the Surveyor Will Ask For

- Dated organizational chart
- Signed job descriptions for each leadership position
- Twelve months of leadership meeting minutes showing decisions and follow-through
- Documentation of the annual review of this policy

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## 1A-02 — Code of Ethics

| Policy # | Owner                   | Review   | Standard area  |
|----------|-------------------------|----------|----------------|
| 1A-02    | Chief Executive Officer | Annually | 1.A Leadership |

### Purpose

To state the ethical standards every person who works for or on behalf of Example Peer Recovery Services, Inc. must follow.

### Policy

All personnel, board members, students, interns, contractors, and volunteers of Example Peer Recovery Services, Inc. conduct themselves according to this Code of Ethics. Each signs an acknowledgment at hire, at any revision, and annually thereafter. Violation is grounds for corrective action up to and including termination and, where required, report to the applicable credentialing or licensing body.

### Procedure

Personnel of Example Peer Recovery Services, Inc. shall:

1. Put the interests, dignity, safety, and self-determination of the persons served first.
2. Serve only within the boundaries of their role, training, credential, and demonstrated competence, and refer when a need exceeds that boundary.
3. Maintain confidentiality in accordance with law and organizational policy, in person, in writing, and online.
4. Refuse any gift, gratuity, loan, or favor from a person served or their family beyond a token of nominal value, and refuse to lend or give money to a person served.
5. Never enter into a romantic, sexual, or financial relationship with a person served, and never with a former person served within the period defined by applicable law or credentialing rules.
6. Disclose any actual, potential, or perceived conflict of interest in writing before it can influence a decision.
7. Neither engage in nor tolerate discrimination or harassment on any basis protected by law.
8. Document truthfully. Never record a service that did not occur, alter a record to conceal an error, or bill for a service not delivered as documented.
9. Represent credentials, certifications, and lived experience accurately.
10. Report suspected abuse, neglect, exploitation, or fraud through the required channels without delay.
11. Cooperate fully and truthfully with any internal review, external audit, licensure inspection, or accreditation survey.

### Evidence the Surveyor Will Ask For

- The signed Code of Ethics Attestation in every personnel file, dated within the last twelve months
- Conflict of interest disclosures on file
- Documentation of any ethics concern raised and how it was resolved

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## 1A-03 — Corporate Compliance Program

| Policy # | Owner                         | Review   | Standard area  |
|----------|-------------------------------|----------|----------------|
| 1A-03    | [FILL IN: Compliance Officer] | Annually | 1.A Leadership |

### Purpose

To detect, correct, and prevent violations of law, regulation, payer requirement, and organizational policy.

### Policy

Example Peer Recovery Services, Inc. maintains an active corporate compliance program with a designated Compliance Officer who has direct access to the Chief Executive Officer and to Board of Directors.

### Procedure

1. The Compliance Officer maintains the written Compliance Plan and reports on it to leadership at least quarterly and to Board of Directors at least annually.
2. All personnel receive compliance training at orientation and annually. Training covers fraud, waste and abuse, false claims, documentation integrity, billing accuracy, and how to report a concern.
3. Any person may report a suspected compliance concern to the Compliance Officer in person, in writing, by phone, or anonymously. The reporting method is posted in the workplace.
4. No person is retaliated against for reporting a concern in good faith. Retaliation is itself a compliance violation and is investigated as one.
5. Every report is logged, investigated, and closed with a written finding and, where warranted, a corrective action with a responsible person and a due date.
6. The Compliance Officer conducts documented internal audits of service documentation and billing no less than quarterly, using the Quality Record Review Tool.
7. Any identified overpayment is reported and repaid to the payer within the timeframe required by that payer.
8. Before hire or engagement, every employee, contractor, and board member is screened against the federal exclusion lists and applicable state exclusion lists, and monthly thereafter. Screening results are documented.

### Evidence the Surveyor Will Ask For

- The written Compliance Plan with a current date
- Compliance training rosters
- The compliance report log with dispositions
- Exclusion-list screening documentation, monthly

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## 1A-04 — Cultural Competency and Health Equity

| Policy # | Owner                   | Review   | Standard area  |
|----------|-------------------------|----------|----------------|
| 1A-04    | Chief Executive Officer | Annually | 1.A Leadership |

### Purpose

To ensure services at Example Peer Recovery Services, Inc. fit the culture, language, identity, and community of the people served.

### Policy

Example Peer Recovery Services, Inc. delivers services in a manner that is responsive to the culture, language, race, ethnicity, religion, age, gender, gender identity, sexual orientation, disability, veteran status, socioeconomic status, and recovery pathway of each person served. A written Cultural Competency and Diversity Plan is maintained, implemented, and reviewed annually.

### Procedure

1. Annually, the Chief Executive Officer reviews the demographics of the people served, of the community in the service area, and of the workforce, and documents any gaps.
2. The Cultural Competency and Diversity Plan sets at least one measurable objective per year addressing an identified gap.
3. All personnel complete cultural competency training at orientation and annually. Training includes recovery-pathway neutrality: no single pathway to recovery is promoted as superior, and a person's chosen pathway, including medication-assisted recovery, is respected.
4. Interpretation and translation are arranged at no cost to the person served when the person's preferred language is not spoken by assigned staff. Family members are not used as interpreters for anything other than social conversation.
5. Written materials given to persons served are provided in plain language and, on request, in the person's preferred language and in an alternate format.
6. Progress against the plan is reported to leadership annually and included in the annual performance analysis.

### Evidence the Surveyor Will Ask For

- The current Cultural Competency and Diversity Plan with this year's objectives
- The demographic comparison that produced them
- Training records
- Documentation of an interpretation arrangement, if one occurred

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_



## 1A-05 — Non-Retaliation and Reporting of Concerns

| Policy # | Owner                   | Review   | Standard area  |
|----------|-------------------------|----------|----------------|
| 1A-05    | Chief Executive Officer | Annually | 1.A Leadership |

### Purpose

To guarantee that any person may raise a concern about Example Peer Recovery Services, Inc. without fear of punishment.

### Policy

No employee, contractor, volunteer, student, person served, family member, or community partner shall be retaliated against for reporting, in good faith, a concern about quality of care, safety, rights, ethics, billing, or compliance, or for participating in an investigation or an accreditation survey.

### Procedure

1. Concerns may be raised to any supervisor, to the Compliance Officer, to the Chief Executive Officer, or to Board of Directors. Contact information for each route is posted where staff and persons served can see it.
2. Concerns may be submitted anonymously.
3. Persons served are additionally informed of their right to contact the applicable licensing authority, protection and advocacy organization, payer, and CARF directly at any time, and are never discouraged from doing so.
4. Retaliation includes termination, demotion, schedule change, reassignment, exclusion, discharge from service, reduction of service, or any other adverse action taken because of a report.
5. An allegation of retaliation is investigated by a person not involved in the alleged retaliation and is reported to Board of Directors.
6. The outcome is documented and, where the reporter is known, communicated to the reporter.

### Evidence the Surveyor Will Ask For

- The posted notice showing reporting routes
- The concern log
- Any retaliation allegation and its investigation file
- Staff able to state, in interview, how they would report a concern

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## 1A-06 — Leadership Succession and Continuity

| Policy # | Owner                   | Review   | Standard area  |
|----------|-------------------------|----------|----------------|
| 1A-06    | Chief Executive Officer | Annually | 1.A Leadership |

### Purpose

To keep Example Peer Recovery Services, Inc. operating and its people served safe when a key leader is suddenly unavailable.

### Policy

Example Peer Recovery Services, Inc. maintains a written succession plan naming, for each leadership position, who acts in that role on an emergency basis and who is being developed for it long term.

### Procedure

1. For each leadership position, the succession plan records: the position, the current incumbent, the designated emergency backup by position, the authorities that transfer, and any credential the backup must already hold.
2. Emergency backups are named for absences beginning immediately and lasting up to thirty days. Absences beyond thirty days are addressed by Board of Directors.
3. Critical operating knowledge - passwords held in the organization's password manager, banking authority, payer portal access, key and alarm codes, vendor contacts, and the location of records - is documented and accessible to at least two people.
4. The plan is reviewed annually and whenever a leadership position changes hands.
5. Development actions for identified successors are documented in the Workforce Development Plan.

### Evidence the Surveyor Will Ask For

- The written succession plan, dated within twelve months
- Evidence that at least two people can access critical systems
- The annual review record

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## Section 1.B — Governance

**What this area is about:** If the organization has a governing board, the board has written duties, meets, keeps minutes, evaluates the CEO, evaluates itself, and manages conflicts of interest. A sole-owner agency documents who performs these governance functions instead.

**Questions a surveyor asks in this area:**

- May I see twelve months of board minutes?
- When did the board last evaluate the CEO?
- How does the board assess its own performance?

**Who gets interviewed:** Governing body, CEO

**Governance standards are consultative/optional in some CARF manuals. Confirm applicability in the purchased manual before building this tab.**

### 1B-01 — Governance

| Policy # | Owner              | Review   | Standard area  |
|----------|--------------------|----------|----------------|
| 1B-01    | Board of Directors | Annually | 1.B Governance |

#### Purpose

To define how Example Peer Recovery Services, Inc. is governed and how the governing authority holds itself accountable.

#### Policy

Example Peer Recovery Services, Inc. is governed by Board of Directors, which holds ultimate responsibility for the organization's mission, financial integrity, legal and ethical conduct, and the quality and safety of services.

#### Procedure

1. The duties of Board of Directors are set out in writing and include: approving the mission; approving the strategic plan and the annual budget; selecting, supporting, and annually evaluating the Chief Executive Officer; approving major policies; reviewing organizational performance and risk; and ensuring legal and ethical operation.
2. Board of Directors meets no less than quarterly. Written minutes record attendance, business conducted, and decisions made, and are retained permanently.
3. Each member completes a conflict of interest disclosure at appointment and annually. A member with a conflict on a matter discloses it and abstains from the discussion and the vote; the abstention is recorded in the minutes.
4. Each member receives a documented orientation covering the mission, the services, the applicable regulations, the financial position, and the member's legal duties.
5. Board of Directors evaluates its own performance in writing at least annually and documents actions arising from that evaluation.
6. The Chief Executive Officer is evaluated in writing at least annually against stated expectations. The evaluation is retained in a confidential file.
7. Where Example Peer Recovery Services, Inc. has no board and is governed by an owner or sole member, that person performs and documents every function above, and the documentation is dated and signed.

#### Evidence the Surveyor Will Ask For

- Written governance duties
- Twelve months of minutes
- Signed conflict of interest disclosures for every member
- The most recent board self-assessment and CEO evaluation

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## Section 1.C — Strategic Integrated Planning

**What this area is about:** A written strategic plan exists that was built from real inputs: what is happening in the environment, what stakeholders expect, financial reality, technology, workforce, and risk. It is reviewed at least annually and the review is documented.

**Questions a surveyor asks in this area:**

- Show me the strategic plan and the minutes where it was reviewed.
- What data went into it?
- What changed as a result?

**Who gets interviewed:** CEO, Governing body

### 1C-01 — Strategic Integrated Planning

| Policy # | Owner                   | Review   | Standard area                     |
|----------|-------------------------|----------|-----------------------------------|
| 1C-01    | Chief Executive Officer | Annually | 1.C Strategic Integrated Planning |

#### Purpose

To ensure Example Peer Recovery Services, Inc. plans its future on evidence rather than assumption.

#### Policy

Example Peer Recovery Services, Inc. maintains a written strategic plan, developed from documented inputs, approved by Board of Directors, and reviewed no less than annually.

#### Procedure

1. Before the plan is written or revised, leadership completes and documents an environmental scan covering at minimum:
  - the expectations and needs of persons served and other stakeholders, from the annual input analysis;
  - the demographics and needs of the communities served, including Guilford, Forsyth, Alamance;
  - the competitive, regulatory, and payer environment;
  - the organization's financial position and projections;
  - technology needs and limitations;
  - workforce supply, including the availability of certified peer support specialists;
  - the risks identified in the Risk Management Plan;
  - the results of the annual performance analysis.
2. The plan states the mission, the vision, and time-limited goals with measurable objectives, a responsible position, and a target date for each.
3. Board of Directors approves the plan and the approval is recorded in the minutes.
4. Progress is reviewed by leadership at least quarterly and by Board of Directors at least annually. Each review is documented, including objectives that were not met and what will change.
5. The plan is communicated to personnel and, in a summary format, to persons served and other stakeholders.

#### Evidence the Surveyor Will Ask For

- The current strategic plan with dates and named owners
- The environmental scan documentation behind it
- Minutes showing approval and at least one documented review
- Evidence it was shared with staff and persons served

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## Section 1.D — Input from Persons Served and Other Stakeholders

**What this area is about:** The organization systematically collects input from the people it serves and from other stakeholders (families, referral sources, payers, staff, community partners), analyzes it at least annually, acts on it, and reports the results back to those stakeholders.

**Questions a surveyor asks in this area:**

- Show me last year's satisfaction results.
- What did you change because of them?
- How did you tell the people served what changed?

**Who gets interviewed:** Persons served, Program Director, QI Coordinator

### 1D-01 — Input from Persons Served and Other Stakeholders

| Policy # | Owner                                      | Review   | Standard area                                        |
|----------|--------------------------------------------|----------|------------------------------------------------------|
| 1D-01    | [FILL IN: Quality Improvement Coordinator] | Annually | 1.D Input from Persons Served and Other Stakeholders |

#### Purpose

To gather, analyze, act on, and report back the views of the people Example Peer Recovery Services, Inc. serves and the people it works with.

#### Policy

Example Peer Recovery Services, Inc. systematically seeks input from persons served and from other stakeholders, analyzes that input at least annually, uses it to improve, and reports the results back to those who gave it.

#### Procedure

1. Input is gathered from at least these groups: persons served, families and natural supports (with the person's consent), personnel, referral sources, payers and funders, and community partners.
2. Methods include an annual written or electronic satisfaction survey, a satisfaction question at discharge, a suggestion route available at all times, and at least one facilitated group or listening session with persons served each year.
3. Surveys are offered in plain language and, on request, in the person's preferred language or an alternate format. Responses may be anonymous.
4. Response rates are tracked. Where a group's response rate falls below the target set in the Performance Measurement Plan, the method of collection is changed and the change is documented.
5. The [FILL IN: Quality Improvement Coordinator] analyzes all input at least annually in a written Input Analysis Report identifying strengths, concerns, and trends by group.
6. The report goes to leadership and Board of Directors, and at least one improvement action is taken from it and tracked to completion.
7. Within ninety days of the analysis, a plain-language summary of what was heard and what changed is provided to persons served, personnel, and other stakeholders, and the distribution is documented.

#### Evidence the Surveyor Will Ask For

- Blank and completed surveys, with response rates
- The written Input Analysis Report
- The improvement action taken from it, with evidence of completion
- The "you said, we did" summary and proof it was distributed

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## Section 1.E — Legal Requirements

**What this area is about:** The organization knows and follows the laws, regulations, licensure and contract obligations that apply to it, and there is a written process for staying current and for reviewing contracts before signing.

**Questions a surveyor asks in this area:**

- What licenses and contracts are you operating under?
- Who tracks renewal dates?
- Show me the current license certificate.

**Who gets interviewed:** CEO, Compliance Officer

### 1E-01 — Legal and Regulatory Compliance

| Policy # | Owner                         | Review   | Standard area          |
|----------|-------------------------------|----------|------------------------|
| 1E-01    | [FILL IN: Compliance Officer] | Annually | 1.E Legal Requirements |

#### Purpose

To ensure Example Peer Recovery Services, Inc. knows, tracks, and meets every legal obligation that applies to it.

#### Policy

Example Peer Recovery Services, Inc. identifies and complies with all applicable federal, state, and local laws and regulations, licensure requirements, payer contract requirements, and accreditation requirements.

#### Procedure

1. The Compliance Officer maintains a Regulatory Obligations Register listing each obligation: the requirement, its source, the position responsible, the renewal or reporting date, and where the evidence of compliance is filed.
2. Licenses, certifications, insurance policies, and contracts are tracked by expiration date, with a reminder no less than sixty days before each.
3. Any contract, lease, or business associate agreement is reviewed by the Chief Executive Officer, and by counsel where the obligation is material, before signature. Signed originals are retained.
4. Mandatory external reports - including but not limited to abuse and neglect reports, incident reports to payers or licensing bodies, and cost or utilization reports - are made within the required timeframe and a copy retained.
5. Changes in law or payer requirement are monitored through payer bulletins, the state licensing authority, and the applicable trade association. Material changes are brought to leadership within thirty days and any policy affected is revised.
6. The register is reviewed and updated at least annually.

#### Evidence the Surveyor Will Ask For

- The Regulatory Obligations Register, current
- Current license and insurance certificates
- Signed contracts and business associate agreements
- Evidence of a policy revised in response to a regulatory change

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## Section 1.F — Financial Planning and Management

**What this area is about:** There is an annual budget built on reasonable assumptions, actual results are compared to budget regularly, there are written internal controls (separation of duties, approval limits), fees are documented, and an external financial review is obtained.

**Questions a surveyor asks in this area:**

- Show me the budget and the monthly comparison to actual.
- Who signs checks and who reconciles the bank statement?
- Show me the most recent external review or audit.

**Who gets interviewed:** CEO, Finance

### 1F-01 — Financial Planning and Budgeting

| Policy # | Owner                   | Review   | Standard area                         |
|----------|-------------------------|----------|---------------------------------------|
| 1F-01    | Chief Executive Officer | Annually | 1.F Financial Planning and Management |

#### Purpose

To plan and monitor the finances of Example Peer Recovery Services, Inc. so that services remain viable and resources are used as intended.

#### Policy

Example Peer Recovery Services, Inc. prepares an annual budget based on documented assumptions, approved before the start of the fiscal year, and compares actual results to budget no less than monthly.

#### Procedure

1. The annual budget is built from: historical utilization and revenue; the current authorization and referral volume; the staffing plan; known rate changes; and the resource needs identified in the strategic, technology, workforce, risk, and accessibility plans. The assumptions are written down alongside the budget.
2. The budget separates revenue by payer and expense by category and includes capital needs.
3. Board of Directors approves the budget before the fiscal year begins; approval is recorded in the minutes.
4. A statement of actual versus budget is prepared monthly and reviewed by the Chief Executive Officer. Variances beyond the threshold set in the budget are explained in writing and acted on.
5. Leadership reviews financial results, including cash position and days of operating cash, at least quarterly, and Board of Directors reviews them at least quarterly.
6. Financial results are considered together with performance and risk information when decisions are made about services, staffing, or capital.
7. An external financial review, audit, or compilation appropriate to the size and requirements of Example Peer Recovery Services, Inc. is obtained annually, and any management letter comments are acted on and tracked.

#### Evidence the Surveyor Will Ask For

- The approved budget with its written assumptions
- Twelve months of budget-to-actual statements
- Minutes showing approval and quarterly review
- The most recent external financial review and the response to any findings

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## 1F-02 — Internal Financial Controls, Fees, and Billing Integrity

| Policy # | Owner                   | Review   | Standard area                         |
|----------|-------------------------|----------|---------------------------------------|
| 1F-02    | Chief Executive Officer | Annually | 1.F Financial Planning and Management |

### Purpose

To protect the assets of Example Peer Recovery Services, Inc. and to ensure every claim submitted is true.

### Policy

Example Peer Recovery Services, Inc. maintains written internal controls that separate duties, limit authority, and require documentation before payment or billing.

### Procedure

1. No single person may initiate, approve, and record the same financial transaction. Where the size of Example Peer Recovery Services, Inc. makes full separation impossible, a compensating control is documented - at minimum, review of the bank statement and reconciliation by a person who does not write checks or make deposits.
2. Expenditures require approval at the level stated in the approval matrix. Expenditures above the Chief Executive Officer's limit require Board of Directors approval.
3. Bank accounts are reconciled monthly by a person other than the person who records deposits, and the reconciliation is signed and dated.
4. A written fee schedule is maintained. Persons served are informed in writing before service of any fee they may owe, of the financial assistance or sliding scale available, and of the fact that services are not denied for inability to pay where a payer or grant covers the service.
5. A claim is submitted only when the service was delivered, was authorized, was documented before submission by the person who delivered it, and matches the documented service, date, duration, location, and rendering provider.
6. Documentation supporting each claim is retained for the period required by the payer, and no less than the period in the Records Retention Schedule.
7. A pre-billing review verifies documentation against the claim. Any claim identified as incorrect after submission is voided or repaid within the payer's required timeframe, and the event is logged with the Compliance Officer.

### Evidence the Surveyor Will Ask For

- The written approval matrix and separation of duties
- Signed monthly bank reconciliations
- The fee schedule and a signed financial disclosure in a record
- Pre-billing review documentation and any repayment record

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_



## Section 1.G — Risk Management

**What this area is about:** There is a written risk management plan that names the organization's actual risks (financial, clinical, reputational, staffing, technology, community-based service delivery), what is being done about each, what insurance is carried, and when it was last reviewed.

**Questions a surveyor asks in this area:**

- What are your top risks and what are you doing about them?
- Show me your insurance declarations page.
- When was the plan last reviewed and by whom?

**Who gets interviewed:** CEO, Program Director

### 1G-01 — Risk Management

| Policy # | Owner                   | Review   | Standard area       |
|----------|-------------------------|----------|---------------------|
| 1G-01    | Chief Executive Officer | Annually | 1.G Risk Management |

#### Purpose

To identify what could harm Example Peer Recovery Services, Inc., the people it serves, or its staff, and to reduce that harm before it occurs.

#### Policy

Example Peer Recovery Services, Inc. maintains a written Risk Management Plan, reviewed and updated no less than annually, covering the identification, evaluation, reduction, and financing of risk.

#### Procedure

1. Risks are identified from at least: incident and grievance data, safety inspections, financial results, staffing and turnover data, technology vulnerabilities, community-based and in-home service delivery, transportation, litigation and claims history, payer audits, and staff and stakeholder input.
2. Each identified risk is recorded in the Risk Register with: a description, the likelihood, the potential severity, the current controls, the additional action required, the responsible position, and the target date.
3. Insurance coverage is reviewed annually against the register, and includes at minimum general liability, professional liability, property, workers' compensation, cyber liability where electronic records are held, and non-owned and hired automobile coverage where staff transport persons served or drive for work.
4. Risks specific to peer-delivered, community-based service are addressed explicitly, including lone-worker safety, transportation, boundary and dual-relationship exposure, staff relapse and wellness, and the safety of persons served during community outings.
5. The plan is reviewed at least annually by leadership, presented to Board of Directors, and the review is documented with the date and the participants.
6. Any incident meeting the definition of a sentinel event triggers an immediate review and an update of the register.

#### Evidence the Surveyor Will Ask For

- The written Risk Management Plan with a date inside twelve months
- The Risk Register with named owners and target dates
- Current insurance declarations pages
- Minutes recording the annual review

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## Section 1.H — Health and Safety

**What this area is about:** Written emergency procedures for every foreseeable emergency; drills that are actually run, documented, and critiqued; documented safety inspections; critical incident reporting and analysis; infection control; first aid and emergency training; and safety procedures for staff working alone in the community or in someone's home.

**Questions a surveyor asks in this area:**

- Show me a year of drill records for every shift and every emergency type.
- Walk me through what happens if a peer specialist feels unsafe at a home visit.
- Show me your incident log and the analysis of trends.

**Who gets interviewed:** Safety Officer, Direct staff, Persons served

### 1H-01 — Health and Safety Management

| Policy # | Owner                     | Review   | Standard area         |
|----------|---------------------------|----------|-----------------------|
| 1H-01    | [FILL IN: Safety Officer] | Annually | 1.H Health and Safety |

#### Purpose

To keep persons served, personnel, and visitors safe in every setting where Example Peer Recovery Services, Inc. delivers service.

#### Policy

Example Peer Recovery Services, Inc. maintains a written Health and Safety Plan and a designated Safety Officer, and conducts documented inspections, drills, training, and incident analysis.

#### Procedure

1. The Safety Officer conducts and documents a safety inspection of each site Example Peer Recovery Services, Inc. controls no less than quarterly, using the Safety Inspection Checklist. Deficiencies are recorded with a correction date and tracked to closure.
2. Exits, egress routes, and fire equipment are kept clear and unobstructed at all times. Fire extinguishers are inspected on the schedule required by the local authority.
3. Emergency contact information, evacuation routes, and the emergency procedures summary are posted where personnel and persons served can see them.
4. First aid supplies are maintained and checked quarterly. Personnel who deliver direct service hold current certification in first aid and CPR unless a written exception is approved by the Chief Executive Officer.
5. Personnel are trained in the health and safety procedures at orientation, annually, and whenever a procedure changes. Training is documented with the topic, date, trainer, and attendee signatures.
6. Safety performance - inspection findings, drill results, incidents, and injuries - is reported to leadership at least quarterly and analyzed annually as part of the performance analysis.
7. Persons served receive an orientation to safety and emergency procedures at admission, documented on the Orientation of Person Served Checklist.

#### Evidence the Surveyor Will Ask For

- Four quarters of completed inspection checklists with corrections closed
- Posted evacuation routes and emergency numbers
- First aid and CPR cards for direct service staff
- Safety training rosters

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## 1H-02 — Emergency Procedures and Drills

| Policy # | Owner                     | Review   | Standard area         |
|----------|---------------------------|----------|-----------------------|
| 1H-02    | [FILL IN: Safety Officer] | Annually | 1.H Health and Safety |

### Purpose

To ensure everyone knows exactly what to do in an emergency, and to prove it by practising.

### Policy

Example Peer Recovery Services, Inc. maintains written procedures for every emergency reasonably foreseeable in its service area and tests them through documented drills.

### Procedure

1. Written procedures exist for at minimum: fire; evacuation and relocation to a designated alternate site; shelter in place; severe weather and natural disaster foreseeable in Guilford, Forsyth, Alamance; utility failure including power, water, heat, and internet; medical emergency including overdose; behavioral or violent incident, including an armed intruder; bomb threat; missing person; motor vehicle accident during service; and death of a person served.
2. Each procedure states who does what, in what order, who is called, how persons served are accounted for, and who notifies families, payers, and licensing bodies.
3. Drills are conducted and documented for each applicable emergency type. Fire and evacuation drills are conducted no less than quarterly, on every shift on which service is delivered, and at least one drill per year is unannounced.
4. Each drill is recorded on the Drill Log with the date, time, type, shift, participants, evacuation or response time, what went wrong, and the corrective action. The corrective action is tracked to closure and verified at the next drill.
5. Staff who deliver service in the community or in a person's home carry the emergency procedures summary and know the local emergency number, the nearest emergency department, and the crisis line for Guilford, Forsyth, Alamance.
6. Naloxone is available and personnel are trained in its use where the population served includes persons at risk of opioid overdose.
7. Procedures are reviewed annually and after any actual emergency.

### Evidence the Surveyor Will Ask For

- The written emergency procedures, all types
- Twelve months of drill logs covering every shift, with corrective actions closed
- Documentation of at least one unannounced drill
- Staff able to describe, in interview, what they would do

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## 1H-03 — Critical Incident Reporting and Analysis

| Policy # | Owner                     | Review   | Standard area         |
|----------|---------------------------|----------|-----------------------|
| 1H-03    | [FILL IN: Safety Officer] | Annually | 1.H Health and Safety |

### Purpose

To capture every serious event, respond to it, learn from it, and prevent the next one.

### Policy

Every critical incident involving a person served, a staff member, a visitor, or the property of Example Peer Recovery Services, Inc. is reported, reviewed, analyzed in aggregate, and acted upon.

### Procedure

1. Reportable incidents include at minimum: death; suicide attempt or self-harm; overdose; allegation or suspicion of abuse, neglect, or exploitation; assault or threat of violence; injury requiring medical attention; medication error or adverse reaction observed; elopement or missing person; motor vehicle accident during service; arrest of a person served during service; property damage or theft; breach of confidentiality; use of any physical intervention; and any event requiring police, fire, or emergency medical response.
2. The staff member involved reports the incident to their supervisor immediately, and no later than the end of that shift, and completes the Critical Incident Report before the end of the next business day.
3. The report records facts only - who, what, when, where, who was notified, what was done - and contains no speculation, blame, or conclusion.
4. External reporting to licensing bodies, payers, adult or child protective services, and law enforcement is made within the timeframe each requires. The Compliance Officer confirms and documents that each required report was made.
5. The Safety Officer reviews every incident within five business days for immediate corrective action and for whether the person served needs an updated crisis plan or person-centered plan.
6. Incidents are logged and aggregated at least quarterly by type, location, staff, time of day, and person served, and analyzed for trends. The analysis, including "no trend identified," is written down.
7. Trends drive an improvement action tracked under the Performance Improvement Plan.
8. The incident report is filed in an administrative incident file. A factual summary of the event is documented in the record of the person served; the incident report form itself is not filed in that record.

### Evidence the Surveyor Will Ask For

- The incident log for twelve months
- Completed incident reports with supervisor review dates
- The quarterly aggregate analysis, in writing
- The improvement action that came from a trend

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## 1H-04 — Infection Prevention and Control

| Policy # | Owner                     | Review   | Standard area         |
|----------|---------------------------|----------|-----------------------|
| 1H-04    | [FILL IN: Safety Officer] | Annually | 1.H Health and Safety |

### Purpose

To prevent the transmission of infectious disease among persons served, personnel, and visitors.

### Policy

Example Peer Recovery Services, Inc. follows standard precautions and applicable public health guidance in all settings, including offices, vehicles, community locations, and the homes of persons served.

### Procedure

1. Personnel are trained at orientation and annually in standard precautions, hand hygiene, respiratory etiquette, safe handling of body fluids, and bloodborne pathogen exposure control.
2. Hand hygiene supplies, gloves, and personal protective equipment appropriate to the service are supplied by Example Peer Recovery Services, Inc. and are carried by staff delivering service in the community.
3. Personnel with a communicable illness that may be transmitted through ordinary contact do not deliver in-person service until they meet the return criteria in this policy or those of the public health authority, whichever is stricter.
4. Any exposure of a staff member or person served to blood or body fluids is reported immediately, documented as a critical incident, and referred for medical evaluation the same day.
5. Sharps and biohazardous waste, if encountered, are not handled by peer support personnel; the person served or the appropriate authority is notified and the event is documented.
6. Vehicles and frequently touched office surfaces are cleaned on a documented schedule.
7. Where a public health emergency is declared, Example Peer Recovery Services, Inc. follows the directives of the public health authority and documents the operational changes made.

### Evidence the Surveyor Will Ask For

- Infection control training records
- Evidence that PPE and hand hygiene supplies are stocked, including in vehicles
- The cleaning schedule
- Any exposure incident and its follow-up

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## 1H-05 — Community, Home Visit, and Lone Worker Safety

| Policy # | Owner                            | Review   | Standard area         |
|----------|----------------------------------|----------|-----------------------|
| 1H-05    | [FILL IN: Program Director name] | Annually | 1.H Health and Safety |

### Purpose

Peer support is delivered where people live their lives. This policy keeps staff safe when they are alone, off site, and in someone else's environment.

### Policy

No staff member of Example Peer Recovery Services, Inc. is required to enter or remain in any location they reasonably believe to be unsafe. Withdrawing from an unsafe situation is never grounds for discipline.

### Procedure

1. Before the first visit to any new location, the assigned staff member and the supervisor complete a Community Visit Safety Plan noting known risks: weapons in the home, unrestrained animals, other occupants, substance use in the home, prior violence, isolated location, poor lighting, and lack of cell coverage.
2. Staff record their itinerary - location, person served, expected arrival and departure - in the shared schedule before leaving, and update it when it changes.
3. A check-in and check-out protocol is used for visits: the staff member notifies the designated contact on arrival and departure. If the check-out is more than thirty minutes overdue and the staff member cannot be reached, the supervisor initiates the escalation sequence in this policy, ending with a request for a welfare check by law enforcement.
4. Staff carry a charged mobile phone at all times during community service. Where coverage is unreliable, the visit is scheduled with a partner or moved to a covered location.
5. Meetings may be relocated to a public place, the office, or a telehealth contact at the discretion of the staff member or the supervisor, and the reason is documented.
6. Two-person visits are used where the safety plan indicates elevated risk, for a first visit to an unknown location where risk is unclear, and at any staff member's request.
7. Staff do not enter a home where an unauthorized or intoxicated person is present and objecting, do not intervene physically in any dispute, and leave immediately if they feel unsafe, reporting the event to the supervisor the same day.
8. Personal vehicles are not used to transport persons served unless the vehicle is documented as insured for that use and the driver is approved under the Transportation policy.
9. Every safety withdrawal, near miss, or threat is recorded as a critical incident and reviewed in supervision.

### Evidence the Surveyor Will Ask For

- Completed Community Visit Safety Plans in the record or the staff file
- The itinerary and check-in log
- Staff able to describe the check-in and escalation steps in interview
- Incident reports for safety withdrawals and how they were handled

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## Section 1.I — Human Resources

**What this area is about:** Written job descriptions; verification of credentials before hire (including the state peer credential); background checks; documented orientation; competency-based training with documented competency, not just attendance; regular documented supervision; annual performance appraisal; and a documented look at workforce needs and succession.

**Questions a surveyor asks in this area:**

- Pull three personnel files. Show me credential verification, orientation, competency, supervision, and the last appraisal in each.
- How do you know a new peer specialist is competent, not just trained?
- Show me the supervision notes for the last six months.

**Who gets interviewed:** HR, Supervisor, Peer Support Specialists

### 1I-01 — Recruitment, Verification, and Hiring

| Policy # | Owner                   | Review   | Standard area       |
|----------|-------------------------|----------|---------------------|
| 1I-01    | Chief Executive Officer | Annually | 1.I Human Resources |

#### Purpose

To ensure every person delivering service for Example Peer Recovery Services, Inc. is qualified, verified, and legally eligible before their first day with a person served.

#### Policy

No person delivers service, alone or otherwise, until every pre-service requirement in this policy is complete and documented.

#### Procedure

1. Every position has a current written job description stating: the title, the reporting line, the minimum qualifications, the required credential, the essential functions, and the physical and travel requirements. The employee signs it at hire and at any revision.
2. Before hire, Example Peer Recovery Services, Inc. verifies and documents on the Credential Verification Log:
  - identity and legal authorization to work;
  - education and any degree claimed, from the source;
  - the state peer credential where required for the role - for Example Peer Recovery Services, Inc. this is NC Certified Peer Support Specialist (NCCPSS) - verified from the issuing registry, with the certificate number and expiration recorded;
  - professional license where required, verified from the licensing board;
  - a criminal background check as required by state law and payer contract;
  - the applicable abuse and neglect registry checks;
  - federal and state exclusion list screening;
  - a driving record check and proof of a valid licence and insurance where the role involves driving;
  - at least two references, documented.
3. Credential and licence expiration dates are tracked and re-verified before expiry. A person whose required credential has lapsed does not deliver service until it is restored, and the removal from service is documented.
4. Lived experience of mental health or substance use recovery is a bona fide qualification for peer support positions and is stated in the job description. A candidate's recovery status is never documented in the personnel file in clinical detail; the file records only that the qualification was met and the credential verified.
5. Contractors, students, interns, and volunteers who have contact with persons served meet the same verification requirements, documented in a file for each.

#### Evidence the Surveyor Will Ask For

- The Credential Verification Log with dated source verifications
- Signed job descriptions in every file
- Background and registry checks dated before the first day of service
- Evidence that an expiring credential was tracked and re-verified

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_



## 1I-02 — Orientation and Training

| Policy # | Owner                            | Review   | Standard area       |
|----------|----------------------------------|----------|---------------------|
| 1I-02    | [FILL IN: Program Director name] | Annually | 1.I Human Resources |

### Purpose

To prepare every person for the work before they do it, and to keep them current.

### Policy

Every employee, contractor, student, intern, and volunteer completes a documented orientation before independent contact with persons served, and completes required training annually thereafter.

### Procedure

1. Orientation is completed within the first thirty days of hire and before independent service delivery, whichever is sooner, and is documented on the Orientation Checklist signed by the employee and the supervisor. Orientation covers at minimum: the mission and program description; the Code of Ethics; rights of persons served and the grievance process; confidentiality, HIPAA; documentation requirements and timeframes; emergency and safety procedures; incident reporting; abuse, neglect, and exploitation reporting duties; infection control; cultural competency; nonviolent practices and de-escalation; boundaries, self-disclosure, and dual relationships; community and lone-worker safety; the person-centered planning process; and the employee's own job description.
2. Annual refresher training covers at minimum: rights, confidentiality, ethics, safety and emergency procedures, incident reporting, abuse and neglect reporting, infection control, cultural competency, de-escalation, and compliance.
3. Additional training is assigned when a new procedure is adopted, when performance or an incident indicates a need, and when the population served changes.
4. Every training is documented with the topic, date, duration, trainer name and qualification, and the signature of each attendee. Attendance alone does not establish competency; see the Competency policy.
5. The [FILL IN: Program Director name] maintains a Training Log per employee showing required trainings, dates completed, and next due dates, and reviews it quarterly for overdue items.

### Evidence the Surveyor Will Ask For

- Signed orientation checklists dated before first independent service
- The training log per employee with no overdue required items
- Sign-in sheets and curricula for each training delivered
- Evidence of training assigned in response to an incident

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## 1I-03 — Competency Assessment

| Policy # | Owner                                       | Review   | Standard area       |
|----------|---------------------------------------------|----------|---------------------|
| 1I-03    | [FILL IN: Supervisor of the peer workforce] | Annually | 1.I Human Resources |

### Purpose

Training proves a person attended. Competency proves they can do the work. CARF asks for the second.

### Policy

Example Peer Recovery Services, Inc. assesses and documents the competency of every person delivering service, at hire and no less than annually, against the competencies required by their role.

### Procedure

1. The Competency Checklist for peer support personnel covers at minimum: engagement and relationship building; strategic and purposeful self-disclosure; person-centered goal setting in the person's own words; recovery and wellness planning; navigating benefits, housing, employment, education, and health care systems; crisis recognition, de-escalation, and warm hand-off; documentation that meets payer and organizational standards; boundaries and dual-relationship management; trauma-informed practice; cultural responsiveness; motivational and person-centered communication; and mandated reporting.
2. Competency is assessed by at least two of: direct observation of service delivery by the supervisor; review of the person's documentation; a structured skills demonstration or role play; and a knowledge check.
3. Direct observation of at least one live or recorded service contact is completed for every direct service employee within ninety days of hire and at least annually thereafter, and is documented with the date, setting, what was observed, and the rating.
4. Each competency is rated as met or not yet met. Any "not yet met" generates a written development action with a target date, and is reassessed. A person is not assigned independent responsibility for a competency rated not yet met.
5. Completed competency assessments are retained in the personnel file and are separate from the annual performance appraisal, though they inform it.

### Evidence the Surveyor Will Ask For

- Completed competency checklists at hire and annually, signed and dated
- Documented direct observation of service delivery
- A "not yet met" item with its development action and reassessment
- Supervisors able to explain how they know a peer specialist is competent

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## 1I-04 — Supervision

| Policy # | Owner                                       | Review   | Standard area       |
|----------|---------------------------------------------|----------|---------------------|
| 1I-04    | [FILL IN: Supervisor of the peer workforce] | Annually | 1.I Human Resources |

### Purpose

To give every person delivering service regular, documented, developmental supervision.

### Policy

Every person delivering direct service receives scheduled, documented supervision from a qualified supervisor.

### Procedure

1. Supervision is provided by [FILL IN: Supervisor of the peer workforce] or a designee who meets the qualification stated in the job description and any payer or state requirement.
2. Frequency: individual supervision no less than twice monthly for the first ninety days of employment and no less than monthly thereafter, or more often where a payer, credentialing body, or performance concern requires it. Group supervision may supplement but does not replace individual supervision.
3. Every supervision session is documented on the Supervision Note with: date, duration, format, participants, cases discussed, guidance given, safety or ethical issues raised, self-disclosure reviewed, action items with due dates, and both signatures.
4. Supervision addresses at minimum: caseload and engagement; progress against person-centered plans; documentation quality; boundaries, self-disclosure, and dual relationships; safety in the community; the staff member's own wellness and recovery supports; and development goals.
5. A staff member may request additional supervision at any time, including after a difficult event, a safety incident, or a personal recovery challenge, without that request being treated as a performance concern.
6. Supervision notes are retained in the personnel file and reviewed by the [FILL IN: Program Director name] quarterly for completeness and timeliness.
7. Where the supervisor is not themselves a certified peer, arrangements are documented for the peer workforce to also access peer-to-peer consultation from a certified peer specialist.

### Evidence the Surveyor Will Ask For

- Twelve months of signed supervision notes for each direct service employee
- Evidence that new employees received twice-monthly supervision in their first ninety days
- Supervision notes that show boundaries and self-disclosure were actually discussed
- The quarterly completeness review

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## 1I-05 — Performance Appraisal, Workforce Planning, and Personnel Records

| Policy # | Owner                   | Review   | Standard area       |
|----------|-------------------------|----------|---------------------|
| 1I-05    | Chief Executive Officer | Annually | 1.I Human Resources |

### Purpose

To evaluate performance fairly, plan for workforce needs, and keep complete, confidential personnel records.

### Policy

Every employee receives a written performance appraisal at least annually, and Example Peer Recovery Services, Inc. annually reviews its workforce needs.

### Procedure

1. The performance appraisal is written, based on the signed job description and the competency assessment, includes input from the supervisor, includes the employee's own comments, sets goals for the coming year, and is signed and dated by both.
2. An appraisal is completed at the end of any introductory period and annually on a schedule tracked by the Chief Executive Officer.
3. Performance concerns are addressed in writing with a stated expectation, the support offered, a timeframe, and a review date.
4. Annually, leadership documents a workforce analysis covering: current and projected staffing needs against service volume; turnover and its reasons; vacancy duration; the local supply of certified peer support specialists; training and development needs; and succession. The analysis feeds the strategic plan and the budget.
5. A career ladder for peer support personnel is defined in writing, showing the steps from entry peer specialist through senior peer specialist and peer supervisor, and the qualification for each step.
6. Personnel files are maintained for every employee, contractor, student, intern, and volunteer, and contain the items on the Personnel File Checklist. Files are kept locked and are accessible only to those with a business need.
7. Medical information, including any accommodation request, and any information related to a person's own recovery history is kept in a separate confidential file, not in the general personnel file.
8. Personnel records are retained after separation for the period in the Records Retention Schedule.

### Evidence the Surveyor Will Ask For

- A signed appraisal dated within twelve months in every file
- The written annual workforce analysis
- The peer career ladder
- A personnel file audited against the checklist, with medical information stored separately

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## Section 1.J — Technology

**What this area is about:** A written technology and systems plan covering the systems in use, who may access what, how data is backed up and tested, what happens when the system is down, and how confidentiality is protected. If information and communication technology (telehealth, texting, video) is used to deliver service, that has its own written procedures.

**Questions a surveyor asks in this area:**

- What happens to service delivery if your EHR is down for two days?
- When did you last test a restore from backup?
- Show me your texting and social media rules for peer staff.

**Who gets interviewed:** CEO, Privacy Officer, Direct staff

### 1J-01 — Technology and System Planning

| Policy # | Owner                   | Review   | Standard area  |
|----------|-------------------------|----------|----------------|
| 1J-01    | Chief Executive Officer | Annually | 1.J Technology |

#### Purpose

To plan, secure, and sustain the technology Example Peer Recovery Services, Inc. depends on to deliver and document service.

#### Policy

Example Peer Recovery Services, Inc. maintains a written Technology and System Plan, reviewed at least annually, covering the systems in use, access, security, backup, continuity, and planned changes.

#### Procedure

1. The plan inventories every system holding organizational or protected health information: the electronic health record (the electronic health record), email, file storage, billing, telephony, the website, and any mobile device used for work.
2. For each system the plan records: what data it holds, who administers it, who may access it, the vendor and support contact, the contract and renewal date, whether a Business Associate Agreement is in place, and the backup arrangement.
3. Access is granted by role, on the least-privilege principle, approved by the supervisor, recorded in the Device and Access Log, and revoked the same day a person separates or changes role.
4. Backups run at least daily. A restore is tested at least annually and the test result documented. An untested backup is treated as no backup.
5. The plan includes the resources needed - replacement cycles, licences, training - and those needs feed the annual budget.
6. Technology needs identified by staff and by persons served, including access to devices and connectivity for people receiving service in the community, are considered in the plan.
7. The plan is reviewed annually by leadership; the review is dated and documented.

#### Evidence the Surveyor Will Ask For

- The written Technology and System Plan, dated within twelve months
- The system inventory with BAAs attached
- A documented backup restore test
- The Device and Access Log showing a same-day revocation on a separation

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## 1J-02 — Information Security and Confidentiality of Data

| Policy # | Owner                      | Review   | Standard area  |
|----------|----------------------------|----------|----------------|
| 1J-02    | [FILL IN: Privacy Officer] | Annually | 1.J Technology |

### Purpose

To protect the confidentiality, integrity, and availability of protected health information held by Example Peer Recovery Services, Inc..

### Policy

Example Peer Recovery Services, Inc. safeguards protected health information in every form - electronic, paper, and spoken - in accordance with HIPAA and applicable state law.

### Procedure

1. Every user has a unique login. Credentials are never shared. Passwords meet the complexity requirement in the Technology Plan and multi-factor authentication is enabled wherever the system supports it.
2. Devices used for work are encrypted, are locked with a passcode, auto-lock after inactivity, and are enrolled in remote wipe where available. Protected health information is not stored on a personal device outside the approved application.
3. Protected health information is transmitted only through encrypted channels. It is not sent in the body of an unencrypted email or a standard text message.
4. Paper records and any printed material containing protected health information are kept in a locked cabinet in a locked room, are never left in a vehicle overnight, and are transported in a closed, opaque container.
5. Access to records is limited to the minimum necessary for the person's role. Access logs in the electronic health record are reviewed at least quarterly for inappropriate access.
6. A suspected or actual breach is reported to the Privacy Officer immediately. The Privacy Officer investigates, documents the risk assessment, and makes any notification to the affected individual, the Secretary, the media, and the payer within the timeframes required by law.
7. All personnel complete privacy and security training at orientation and annually.
8. Devices and media are sanitized or destroyed before disposal, and the destruction is documented.

### Evidence the Surveyor Will Ask For

- Privacy and security training records
- Quarterly access log reviews
- The breach log and any completed risk assessment
- Evidence of device encryption and documented media destruction

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## 1J-03 — Social Media, Mobile Devices, and Electronic Communication

| Policy # | Owner                            | Review   | Standard area  |
|----------|----------------------------------|----------|----------------|
| 1J-03    | [FILL IN: Program Director name] | Annually | 1.J Technology |

### Purpose

Peer support is relational, and peers and the people they serve often move in the same online communities. This policy sets the boundary.

### Policy

Personnel of Example Peer Recovery Services, Inc. do not form or maintain personal online relationships with persons served, and use only approved channels to communicate with them about service.

### Procedure

1. Personnel do not accept, send, or respond to friend, follow, connection, or direct-message requests from a current person served on a personal account. A request received is declined, and the fact is disclosed in the next supervision.
2. Where a person served is already a personal online contact from the staff member's own recovery community, the staff member discloses this in writing on the Dual Relationship Disclosure before or immediately upon assignment, and the supervisor documents the management plan, which may include reassignment.
3. Personnel never post anything that identifies a person served, never post photographs taken during service, and never confirm or deny that any individual receives services from Example Peer Recovery Services, Inc., on any platform, even where the person served has posted about it themselves.
4. Personnel do not respond publicly to an online review or comment about Example Peer Recovery Services, Inc. from a person served. Such comments are referred to the Chief Executive Officer.
5. Communication with a person served about scheduling or service uses the organizational phone, the organizational email, or the the electronic health record portal. Where texting is used, the person served has given written consent that acknowledges the risks, and the content is limited to scheduling and non-clinical logistics.
6. Every communication with or about a person served that is substantive to the service is documented in the record.
7. Personal use of mobile devices is not permitted during a service contact.
8. Personnel who identify publicly as being in recovery may do so on their own accounts. Where they also identify their employment with Example Peer Recovery Services, Inc., they include a statement that views are their own and post nothing that could identify a person served.

### Evidence the Surveyor Will Ask For

- The signed policy acknowledgment in every personnel file
- Signed texting consents in records where texting is used
- Dual Relationship Disclosures and their management plans
- Supervision notes recording a declined online contact

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## 1J-04 — Information and Communication Technology in Service Delivery (Telehealth)

| Policy # | Owner                            | Review   | Standard area  |
|----------|----------------------------------|----------|----------------|
| 1J-04    | [FILL IN: Program Director name] | Annually | 1.J Technology |

### Purpose

To deliver service by video, telephone, or other technology at the same standard as in person.

**Note:** Example Peer Recovery Services, Inc. does not currently deliver service using information and communication technology. This policy is adopted in advance and applies from the first such contact.

### Policy

Where Example Peer Recovery Services, Inc. delivers any part of its service using information and communication technology, that delivery meets the same standards for privacy, consent, documentation, competency, and quality as in-person service.

### Procedure

1. The person served gives informed consent specific to technology-delivered service before the first such contact. The consent covers: what the technology is, its benefits and limits, the risk that a transmission could be intercepted, the right to refuse or to stop at any time without effect on services, what happens if the connection fails, and how emergencies are handled.
2. Before each technology-delivered contact the staff member confirms the person's identity, the person's physical location at that moment, and a call-back number, and records them in the note.
3. An emergency plan is established for each person receiving technology-delivered service, recording the address where they will normally be, the nearest emergency services, and an emergency contact, so help can be dispatched if needed.
4. Only platforms approved by Example Peer Recovery Services, Inc. and covered by a Business Associate Agreement are used. Personal consumer accounts are not used.
5. Staff deliver technology-based service from a private location where the session cannot be overheard, using a headset, with no other person present unless the person served has consented.
6. Staff are trained and assessed as competent in the platform and in delivering peer support remotely before doing so.
7. Documentation states the modality, the location of each party, who was present, and that consent was in place.
8. If the connection drops and cannot be restored, the staff member telephones the person, documents the interruption, and does not bill for time not delivered.

### Evidence the Surveyor Will Ask For

- Signed technology-specific consents
- Notes recording modality and the location of both parties
- The emergency plan for a person served remotely
- The BAA for the platform in use

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_



## Section 1.K — Rights of Persons Served

**What this area is about:** Rights are written, given to and explained to every person served in a way they can understand, and posted. There is a written grievance/complaint process with timeframes and an appeal, a grievance log, informed consent, and confidentiality protections. Rights are reviewed annually for whether they were actually honored.

**Questions a surveyor asks in this area:**

- Ask a person served: do you know how to file a complaint?
- Show me the grievance log and what happened with each one.
- Show me signed consents in three records.

**Who gets interviewed:** Persons served, Direct staff, Privacy Officer

### 1K-01 — Rights of the Persons Served

| Policy # | Owner                            | Review   | Standard area                |
|----------|----------------------------------|----------|------------------------------|
| 1K-01    | [FILL IN: Program Director name] | Annually | 1.K Rights of Persons Served |

#### Purpose

To state the rights of every person served by Example Peer Recovery Services, Inc. and to ensure those rights are honored in practice.

#### Policy

Every person served by Example Peer Recovery Services, Inc. has the rights listed below. The rights are given and explained to each person at admission in a manner they can understand, are posted where persons served can see them, and are honored by all personnel.

#### Procedure

Every person served has the right to:

1. Be treated with dignity and respect, and to be free from abuse, neglect, exploitation, retaliation, humiliation, and financial exploitation.
2. Be free from restraint or seclusion used for coercion, discipline, convenience, or retaliation. Example Peer Recovery Services, Inc. prohibits restraint, seclusion, and every form of isolation, coercion, and physical intervention used to control a person's movement, in all of its programs, without exception.
3. Receive services in the least restrictive, most integrated setting appropriate, and to have services delivered without discrimination on any basis protected by law.
4. Participate in and consent to their own person-centered plan, to have their goals stated in their own words, and to refuse any service or activity without losing other services or being punished.
5. Choose their own recovery pathway, including the use of medication for a mental health or substance use condition, without pressure to adopt any particular pathway.
6. Confidentiality of their information, and to be told the limits of confidentiality before they share.
7. Access their own record in accordance with law, and to request an amendment.
8. Be informed before service of any fee they may be responsible for.
9. Have their own money and property, and to have Example Peer Recovery Services, Inc. keep no control over it.
10. Voice a complaint or grievance without retaliation, and to be told how, at admission and at any time on request.
11. Contact and be visited by anyone of their choosing, including legal counsel, an advocate, a family member, clergy, the state licensing authority, the protection and advocacy agency, the payer, and CARF, at any time, in private.
12. Be told who is providing their service, that person's role and credential, and to request a different worker.
13. Give or withhold consent to be photographed, recorded, or observed, and to be involved in research or training, without effect on services.
14. Reasonable accommodation of a disability and communication in the person's preferred language and format.
15. Be informed of these rights in writing at admission and on request, and to receive a copy.

Rights are reviewed with the person at admission, documented on the Orientation of Person Served Checklist with the person's signature, and reviewed again at least annually.

**Evidence the Surveyor Will Ask For**

- The rights statement posted and in the handbook
- A signed rights acknowledgment in every record
- Persons served able to say, in interview, that they know their rights
- The annual review of whether rights were honored, in writing

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## 1K-02 — Grievances, Complaints, and Appeals

| Policy # | Owner                            | Review   | Standard area                |
|----------|----------------------------------|----------|------------------------------|
| 1K-02    | [FILL IN: Program Director name] | Annually | 1.K Rights of Persons Served |

### Purpose

To give every person served a fair, timely, documented way to raise a problem and get an answer.

### Policy

Example Peer Recovery Services, Inc. maintains a written grievance procedure available to every person served, their family or guardian with consent, and any other stakeholder. No person is retaliated against for filing.

### Procedure

1. A concern may be raised verbally or in writing, to any staff member, anonymously, or directly to the [FILL IN: Program Director name] or the Chief Executive Officer. Assistance to write it down is offered and is provided by a person who is not the subject of the grievance.
2. Informal resolution is offered first, at the point of service, within two business days. If resolved, the resolution is recorded on the Grievance Log.
3. A formal grievance is acknowledged in writing within three business days of receipt.
4. The formal grievance is investigated by a person not involved in the matter. The person served may present information and may bring a support person or advocate.
5. A written decision, stating the finding, the reason, and any action taken, is issued to the person served within thirty calendar days of receipt. If more time is needed, the person is told in writing before day thirty, with the reason and the new date.
6. The person served may appeal the decision to the Chief Executive Officer within fifteen business days. Where the Chief Executive Officer decided the original grievance, the appeal goes to Board of Directors. A written appeal decision is issued within thirty calendar days and is final within Example Peer Recovery Services, Inc..
7. Every written decision informs the person of their right to take the matter to the payer, the state licensing authority, the protection and advocacy organization, and CARF, and provides the current contact information for each.
8. Services are not reduced, suspended, or terminated because a person filed a grievance.
9. All grievances, formal and informal, are entered on the Grievance Log with the date received, nature, who investigated, the outcome, the date closed, and any systemic action.
10. The log is analyzed at least quarterly for trends, and the analysis is part of the annual performance analysis.

### Evidence the Surveyor Will Ask For

- The written procedure, posted and in the handbook, in plain language
- The Grievance Log for twelve months, including informal concerns
- One complete grievance file with the acknowledgment, investigation, and written decision, showing the timeframes were met
- The quarterly trend analysis

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## 1K-03 — Confidentiality and Release of Information

| Policy # | Owner                      | Review   | Standard area                |
|----------|----------------------------|----------|------------------------------|
| 1K-03    | [FILL IN: Privacy Officer] | Annually | 1.K Rights of Persons Served |

### Purpose

To protect what people tell Example Peer Recovery Services, Inc. and to release it only with authority.

### Policy

Information about a person served is confidential and is disclosed only with a valid written authorization from the person or their legal representative, or where disclosure is required or permitted by law.

### Procedure

1. At admission, and before the person shares personal information, staff explain in plain language what will be kept confidential and the specific limits: risk of harm to self or others, suspected abuse or neglect of a child or vulnerable adult, court order, and required reporting.
2. A Release of Information is valid only if it is written, signed and dated by the person served or their legal representative, and states: who is releasing, who is receiving, the specific information, the specific purpose, an expiration date or event, the right to revoke in writing, and the fact that the receiving party may re-disclose unless prohibited.
3. A blanket or open-ended release is not accepted. A release is not a condition of receiving service.
4. Each disclosure is logged with the date, what was disclosed, to whom, and under which authority.
5. Only the minimum necessary information is disclosed.
6. The person served may revoke a release at any time in writing, except as to information already released. Revocation is documented and acted on the same day.
7. Staff do not discuss a person served in any public area, in a vehicle with another person present, or in any online space.
8. Example Peer Recovery Services, Inc. does not hold records identifying a person as having a substance use disorder within the meaning of 42 CFR Part 2. If that changes, Part 2 procedures are adopted before the first such record is created.
9. Records of persons served are retained and destroyed per the Records Retention Schedule, and destruction is documented.

### Evidence the Surveyor Will Ask For

- Signed, complete, non-expired releases in records
- The disclosure log
- A documented revocation, if one occurred
- Staff able to state the limits of confidentiality in interview

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## 1K-04 — Informed Consent for Services

| Policy # | Owner                            | Review   | Standard area                |
|----------|----------------------------------|----------|------------------------------|
| 1K-04    | [FILL IN: Program Director name] | Annually | 1.K Rights of Persons Served |

### Purpose

To make sure people know what they are agreeing to before service begins.

### Policy

Services are provided only with the informed, written consent of the person served or their legal representative, freely given, and revocable at any time.

### Procedure

1. Before the first service, staff explain and give the person in writing: what the service is and is not; that peer support is a non-clinical, mutual, recovery-oriented service and is not therapy, medical treatment, case management, or legal advice; who will provide it and with what credential; the expected frequency and duration; the voluntary nature of participation; the right to refuse any part; any fee; the limits of confidentiality; the grievance process; and the potential benefits and limits of the service.
2. The explanation is given in plain language and, on request, in the person's preferred language or an alternate format. Comprehension is confirmed by asking the person to say back what they understood, and that confirmation is noted.
3. The person signs and dates the Consent for Services. A copy is offered to the person and the original is retained in the record.
4. Where the person has a legal guardian or representative, the representative signs, the legal authority is verified and a copy of it retained, and the person served is still involved in the discussion to the greatest extent possible.
5. Consent is renewed at least annually and whenever the nature of the service materially changes.
6. Separate written consent is obtained for: technology-delivered service, texting, transportation, photography or recording, participation in research or training, and any disclosure of information.
7. Withdrawal of consent is honored immediately, documented, and does not affect the person's access to other services or their right to return.

### Evidence the Surveyor Will Ask For

- Signed, dated consents in every record, renewed within twelve months
- Separate consents for texting, transport, and technology where those occur
- Guardianship documentation where applicable
- A person served able to describe what they agreed to

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## Section 1.L — Accessibility

**What this area is about:** A written accessibility plan that identifies real barriers in eight areas - architectural, environmental, attitudinal, financial, employment, communication, transportation, and digital/technology - names an action and a responsible person for each, and is reviewed annually with progress documented.

**Questions a surveyor asks in this area:**

- Show me your accessibility plan and this year's progress.
- What barrier did you remove in the last twelve months?
- How would a person who is deaf access your service?

**Who gets interviewed:** CEO, Persons served, Direct staff

### 1L-01 — Accessibility

| Policy # | Owner                   | Review   | Standard area     |
|----------|-------------------------|----------|-------------------|
| 1L-01    | Chief Executive Officer | Annually | 1.L Accessibility |

#### Purpose

To find and remove the barriers that stop people from getting or using the services of Example Peer Recovery Services, Inc..

#### Policy

Example Peer Recovery Services, Inc. maintains a written Accessibility Plan identifying barriers, the action to remove or reduce each, the responsible position, and a target date. The plan is reviewed and updated at least annually with documented progress.

#### Procedure

1. Barriers are identified annually in eight areas, using the Accessibility Barrier Survey, input from persons served and personnel, and observation:
  - **Architectural** - parking, entrance, ramps, door width and hardware, restrooms, counter heights, interior circulation.
  - **Environmental** - lighting, noise, temperature, seating, scent, sensory load in group and meeting spaces.
  - **Attitudinal** - stigma toward mental health, substance use, criminal legal history, homelessness, or disability, in staff and in community partners.
  - **Financial** - fees, transportation cost, cost of documents or copies, the cost of participating in a community activity.
  - **Employment** - accessibility of the hiring process and of the workplace to people with disabilities and people in recovery, including people with a criminal legal history.
  - **Communication** - reading level of documents, plain language, languages spoken, interpretation, sign language, alternate formats, hearing loop.
  - **Transportation** - availability, cost, routes, and timing of transport to services and to community destinations in Guilford, Forsyth, Alamance.
  - **Digital and technology** - website accessibility, device and internet access among persons served, usability of the portal, digital literacy.
2. Each barrier is entered in the plan with an action, a responsible position, a target date, and the resources needed. Resource needs feed the annual budget.
3. Where a barrier cannot be removed, the plan records the interim accommodation that will be provided instead.
4. A request for accommodation from a person served, a family member, or an employee is responded to within five business days, documented, and, where granted, implemented; where denied, the reason and the alternative offered are documented.
5. Progress is reviewed at least annually by leadership, documented, and reported to Board of Directors and in the annual performance analysis.

#### Evidence the Surveyor Will Ask For

- The written Accessibility Plan covering all eight areas, dated within twelve months
- Documented progress, including at least one barrier removed or reduced this year
- A completed accommodation request and its outcome
- The barrier survey that produced the plan

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## Section 1.M — Performance Measurement and Management

**What this area is about:** The organization decides in writing what it will measure, why, how, how often, and what the target is - covering both business function and service delivery, and covering effectiveness, efficiency, service access, and satisfaction. Data is actually collected and reported on schedule.

**Questions a surveyor asks in this area:**

- What exactly do you measure and what is your target?
- Show me the data for the last four quarters.
- How do you define a successful outcome for a person in peer support?

**Who gets interviewed:** QI Coordinator, CEO, Program Director

### 1M-01 — Performance Measurement and Management

| Policy # | Owner                                      | Review   | Standard area                              |
|----------|--------------------------------------------|----------|--------------------------------------------|
| 1M-01    | [FILL IN: Quality Improvement Coordinator] | Annually | 1.M Performance Measurement and Management |

#### Purpose

To decide in advance what Example Peer Recovery Services, Inc. will measure, and to collect it reliably.

#### Policy

Example Peer Recovery Services, Inc. maintains a written Performance Measurement and Management Plan defining what is measured, why, how, by whom, how often, and what the target is, covering both business function and service delivery.

#### Procedure

1. For each measure the plan records, on a Measure Definition Sheet: the name; what it tells us; the exact numerator and denominator; the data source; who collects it; the frequency; the target; and how it will be reported.
2. Service delivery measures cover, at minimum, these four domains:
  - **Effectiveness** - did the person achieve what they came for. Example measures: percentage of persons served who achieved at least one person-centered plan goal; percentage who report improved quality of life; percentage who acquired or maintained housing, employment, education, or a natural support.
  - **Efficiency** - resources used per result. Example measures: average length of service to goal achievement; average cost per person served; caseload per peer specialist; percentage of scheduled contacts kept.
  - **Service access** - can people get in. Example measures: days from referral to first contact; days from first contact to person-centered plan; percentage of referrals engaged; number and reason for referrals declined.
  - **Satisfaction** - what people say. Example measures: overall satisfaction of persons served; percentage who would recommend the service; stakeholder satisfaction; response rate.
3. Business function measures cover at minimum: budget-to-actual variance; claim denial rate and reason; documentation timeliness; staff turnover; training and credential currency; incident rate; and grievance rate.
4. Data is collected on the stated schedule and reported on the Quarterly Performance Dashboard.
5. Data integrity is checked at least annually: a sample is traced back to the source record and any discrepancy is corrected and documented.
6. The plan is reviewed at least annually. Measures that no longer inform a decision are retired, in writing, with the reason.

#### Evidence the Surveyor Will Ask For

- The written plan with a definition sheet for every measure, including targets
- Four quarters of actual data on the dashboard
- The data integrity check
- Staff able to say what their program measures and how it is doing

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_



## Section 1.N — Performance Improvement

**What this area is about:** The measured data is analyzed at least annually in a written performance analysis, conclusions are drawn, improvement actions are taken and tracked, and the results are shared with staff, persons served, and other stakeholders in a format they can understand.

**Questions a surveyor asks in this area:**

- Show me last year's written performance analysis.
- Name one improvement project and show me the before and after numbers.
- How did the people you serve find out about these results?

**Who gets interviewed:** QI Coordinator, CEO, Direct staff, Persons served

### 1N-01 — Performance Improvement

| Policy # | Owner                                      | Review   | Standard area               |
|----------|--------------------------------------------|----------|-----------------------------|
| 1N-01    | [FILL IN: Quality Improvement Coordinator] | Annually | 1.N Performance Improvement |

#### Purpose

To turn the data into change, and to tell people what changed.

#### Policy

Example Peer Recovery Services, Inc. analyzes its performance data at least annually in writing, draws conclusions, takes improvement action, and communicates the results to personnel, persons served, and other stakeholders.

#### Procedure

1. At least annually the [FILL IN: Quality Improvement Coordinator] prepares the written Annual Performance Analysis Report containing:
  - the results for every measure in the Performance Measurement Plan, against target;
  - comparison to the prior period, and to any external benchmark used;
  - the results of the input analysis, the grievance analysis, the incident analysis, the record review, and the accessibility review;
  - written conclusions about what is working and what is not, including where a target was met;
  - the improvement actions to be taken, each with a responsible position and a target date;
  - the status of the actions from the previous year's report.
2. Where a target is missed, the analysis states the probable cause and the corrective action. "No action" is an acceptable conclusion only where it is stated and justified in writing.
3. Improvement projects use a documented method - a stated problem, a measurable aim, the change tested, the data before and after, and the decision to adopt, adapt, or abandon - recorded on the PI Project Charter.
4. The report is presented to leadership and to Board of Directors, and the presentation is recorded in the minutes.
5. Within ninety days of completion, a plain-language summary of the results is shared with personnel, with persons served, and with other stakeholders in a format each can use. The distribution is documented.
6. The report informs the next strategic plan, budget, risk management plan, and training plan; those links are stated in the report.

#### Evidence the Surveyor Will Ask For

- The written Annual Performance Analysis Report, dated
- A completed PI project with before and after data
- Minutes showing the report was presented
- The plain-language summary and proof it reached persons served and staff

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## Section 2.A — Program/Service Structure

**What this area is about:** A written program description that says who the program serves, what it does, where and when, who delivers it and with what qualifications, and how it connects people to community resources. Services are person-centered and recovery-oriented, and staffing is sufficient for the people served.

**Questions a surveyor asks in this area:**

- Show me the written program description.
- Does what I am seeing on site match what this says?
- How many people can one peer specialist carry, and how did you decide that?

**Who gets interviewed:** Program Director, Direct staff

### 2A-01 — Program Description and Service Structure

| Policy # | Owner                            | Review   | Standard area                 |
|----------|----------------------------------|----------|-------------------------------|
| 2A-01    | [FILL IN: Program Director name] | Annually | 2.A Program/Service Structure |

#### Purpose

To describe in writing exactly what Community Integration is, so that what is written matches what a surveyor sees.

#### Policy

Example Peer Recovery Services, Inc. maintains a written program description for each program it operates. The description is accurate, current, given to persons served and referral sources, and reviewed at least annually.

#### Procedure

1. The program description states: the program name; the population served, including age range and eligibility; the purpose and philosophy; the specific services offered; the settings where service is delivered - the agency office, community locations, the homes of persons served; the hours and days of operation; the geographic area served - Guilford, Forsyth, Alamance; the staffing, including the credential of each role; the expected frequency, duration, and intensity of service; the expected length of service; the outcomes the program seeks; and how the program connects people to resources outside itself.
2. The description states that services are recovery-oriented, person-centered, trauma-informed, and delivered by certified peer support specialists with lived experience.
3. The description is reviewed against actual practice at least annually and whenever the service changes. Where practice has drifted from the description, either the practice or the description is corrected, and the correction is documented.
4. The description is provided to every person served at admission, to referral sources, and to payers, and is available on request.
5. Staffing is determined by a written staffing plan that states the maximum caseload per peer support specialist, the basis for that number, the supervisory ratio, and how coverage is provided during absence. The plan is reviewed at least annually against outcome, incident, satisfaction, and turnover data.
6. Services are delivered in the most integrated community settings appropriate to each person's goals, rather than in segregated settings, wherever the person chooses.

#### Evidence the Surveyor Will Ask For

- The current written program description
- The staffing plan with the caseload maximum and its rationale
- Evidence the description was given to persons served
- Agreement between the description and what is observed on site

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## 2A-02 — Community Resources and Coordination of Care

| Policy # | Owner                            | Review   | Standard area                 |
|----------|----------------------------------|----------|-------------------------------|
| 2A-02    | [FILL IN: Program Director name] | Annually | 2.A Program/Service Structure |

### Purpose

Peer support works by connecting people to what exists in their community. This policy makes that systematic.

### Policy

Example Peer Recovery Services, Inc. maintains current knowledge of community resources and coordinates with the other providers and supports in each person's life, with that person's consent.

### Procedure

1. A Community Resource Directory covering Guilford, Forsyth, Alamance is maintained and reviewed at least every six months for accuracy. It covers at minimum: crisis and warm lines; emergency and inpatient services; housing and homeless services; food; benefits and entitlements offices; primary and behavioral health care; medication for addiction treatment providers; employment and vocational rehabilitation; education and literacy; legal aid; transportation; childcare; recovery community organizations and mutual aid meetings of multiple pathways; faith communities; and recreation.
2. Referral to an outside resource is documented in the record: what was referred, why, when, to whom, and what happened.
3. With the person's written consent, Example Peer Recovery Services, Inc. coordinates with the person's other providers, including their treatment team, prescriber, probation officer, housing worker, or family, and the coordination is documented.
4. Where a person served is receiving services from another provider, Example Peer Recovery Services, Inc. clarifies in writing which provider is responsible for what, so the person is not caught between them.
5. Formal relationships with key partners are documented in a memorandum of understanding stating the referral pathway and the contact for each side, where the partner will agree to one.
6. Gaps in community resources identified by staff or persons served are recorded and reported annually to leadership as an input to the strategic plan and to community advocacy.

### Evidence the Surveyor Will Ask For

- The Community Resource Directory, dated within six months
- Documented referrals and their outcomes in records
- Signed consents supporting coordination
- Any memoranda of understanding

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## Section 2.B — Screening and Access to Services

**What this area is about:** Written eligibility criteria, a documented screening process with timeframes, what happens when someone is not eligible or must wait, and a documented orientation for every person entering the program that covers rights, grievance, confidentiality, fees, and what to expect.

**Questions a surveyor asks in this area:**

- How long between the referral and the first contact? Prove it.
- Show me a signed orientation checklist in a record.
- What do you do when someone does not meet criteria?

**Who gets interviewed:** Program Director, Direct staff, Persons served

### 2B-01 — Screening, Eligibility, and Access to Services

| Policy # | Owner                            | Review   | Standard area                        |
|----------|----------------------------------|----------|--------------------------------------|
| 2B-01    | [FILL IN: Program Director name] | Annually | 2.B Screening and Access to Services |

#### Purpose

To get people into service quickly and fairly, and to be honest with those the program cannot serve.

#### Policy

Example Peer Recovery Services, Inc. has written eligibility criteria and a documented screening process with defined timeframes, applied consistently and without discrimination.

#### Procedure

1. Written eligibility criteria for Community Integration state the age range, the presenting need, the geographic area, the payer or funding requirements, and any exclusion. Exclusions are based only on the program's demonstrated inability to meet the need safely, never on a protected characteristic, on recovery pathway, on medication for addiction treatment, on criminal legal history alone, or on prior discharge.
2. A referral is logged on receipt with the date, source, and referral reason.
3. Initial contact with the referred person is attempted within 3 business days of receipt of the referral. Every attempt is logged. After three documented attempts across different days and times, the referral source is notified and the referral is closed.
4. The screening determines eligibility and urgency and is documented on the Screening and Referral Form. Anyone screening as being at imminent risk is connected to crisis services immediately and not placed on a waiting list.
5. Where the person is eligible and capacity exists, the first service contact is scheduled within 3 business days of the screening.
6. Where capacity does not exist, the person is placed on a waiting list with their consent, is told the expected wait, is contacted no less than every fourteen days while waiting, is offered interim resources, and is told where else they may go. All of this is logged on the Waitlist Log.
7. Where the person is not eligible, they are told the reason in plain language, given at least two specific alternative resources with contact information, and offered help to make that connection. The referral out is logged.
8. Access data - referrals received, days to first contact, days to first service, engagement rate, waitlist length, and reasons for non-acceptance - is reported quarterly under the Performance Measurement Plan and analyzed for barriers.

#### Evidence the Surveyor Will Ask For

- The written eligibility criteria
- The referral log showing dates and days-to-contact
- The waitlist log with contacts every fourteen days
- The referral-out log showing where declined people were sent

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## 2B-02 — Orientation of the Person Served

| Policy # | Owner                            | Review   | Standard area                        |
|----------|----------------------------------|----------|--------------------------------------|
| 2B-02    | [FILL IN: Program Director name] | Annually | 2.B Screening and Access to Services |

### Purpose

To make sure every person entering service knows their rights and what to expect, and to prove it.

### Policy

Every person admitted to Community Integration receives a documented orientation at or before the first service contact, in a manner and language they understand.

### Procedure

1. Orientation covers at minimum: the program description and what the service is and is not; the rights of persons served, in writing and explained; the grievance and appeal process, including the outside bodies they may contact; confidentiality and its limits; informed consent; any fee and the financial assistance available; the person-centered planning process and their role in it; how to reach staff, including after hours and in a crisis; what to do in an emergency at each site; safety and evacuation procedures; the expectations of participation; how to request a different worker; the smoking, weapons, and substance policies; and how to give feedback.
2. Orientation is delivered in plain language, at a reading level accessible to the person, and, on request, in the person's preferred language or an alternate format. A support person or advocate may attend.
3. Comprehension is confirmed by asking the person to state back the key points, particularly how to file a grievance. The confirmation is noted.
4. The person signs the Orientation of Person Served Checklist. The person receives a copy of the checklist, the rights statement, the grievance procedure, and the program description. If the person declines to sign, that is documented with the reason and the orientation still occurs.
5. Orientation is repeated at least annually, and whenever the program, the rights, or the grievance process materially changes.
6. Where the person has a guardian, the guardian also receives orientation, and the person served is oriented to the greatest extent possible regardless.

### Evidence the Surveyor Will Ask For

- A signed, dated orientation checklist in every record, at admission and annually
- The handbook or packet given to persons served
- Persons served able to state, in interview, how to file a grievance
- Evidence of an orientation delivered in another language or format

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## Section 2.C — Person-Centered Plan

**What this area is about:** Each person has an individualized plan built with them, in their own words, reflecting their strengths, needs, abilities and preferences, with goals and measurable objectives, the specific services and who provides them, target dates, and the person's signature. The plan is reviewed on a stated schedule and updated when things change.

**Questions a surveyor asks in this area:**

- Read me a goal in the person's own words.
- Show me where the person signed and dated the plan.
- Show me the review that was due last month.
- Does this progress note connect to a goal on this plan?

**Who gets interviewed:** Persons served, Direct staff, Supervisor

### 2C-01 — Person-Centered Planning

| Policy # | Owner                            | Review   | Standard area            |
|----------|----------------------------------|----------|--------------------------|
| 2C-01    | [FILL IN: Program Director name] | Annually | 2.C Person-Centered Plan |

#### Purpose

The person-centered plan is the single document a surveyor will read most closely. This policy defines how it is built, with the person, not for them.

#### Policy

Every person served by Example Peer Recovery Services, Inc. has an individualized, person-centered plan developed with them, in their own words, based on their own goals, and signed by them.

#### Procedure

1. The plan is developed with the person within 30 days of admission. Anyone the person invites - family, natural supports, other providers - participates with the person's consent.
2. Before goals are written, the peer support specialist documents the person's **strengths, needs, abilities, and preferences** in the person's own words, along with what the person says has helped and has not helped in the past, their culture and identity, their chosen recovery pathway, and what a good life looks like to them.
3. Each goal is:
  - stated in the person's own words, in the first person, and recorded in quotation marks;
  - about something the person actually wants, not something the program or a payer wants for them;
  - supported by measurable objectives that state what will be different, by how much, and by when;
  - matched to specific services and the position responsible for each;
  - given a target date.
4. The plan states the frequency, duration, intensity, and location of each service.
5. The plan identifies the person's natural supports and community connections and how the plan will strengthen them, so that the person's support does not depend on paid staff alone.
6. The plan includes what the person wants to happen in a crisis, cross-referenced to their Personal Safety and Crisis Plan.
7. The plan states, from the beginning, what the person wants their life to look like when services end - the transition goal. See 2D-01.
8. The person signs and dates the plan. A copy is offered to the person and the offer is documented. If the person declines to sign, that is documented with the reason and services continue.
9. Where a legal guardian must also sign, the person served still participates and their voice is still recorded in their own words.
10. Every service note connects to a goal on the current plan. A service that does not connect to a goal is either a reason to update the plan or a reason not to bill it.

#### Evidence the Surveyor Will Ask For

- A plan in every record, signed and dated by the person served

- Goals in the person's own words, in quotation marks - the surveyor will read them aloud
- Objectives that are actually measurable
- Progress notes that link to a goal on the current plan

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## 2C-02 — Plan Review and Documentation of Services

| Policy # | Owner                                       | Review   | Standard area            |
|----------|---------------------------------------------|----------|--------------------------|
| 2C-02    | [FILL IN: Supervisor of the peer workforce] | Annually | 2.C Person-Centered Plan |

### Purpose

To keep the plan alive and to document services in a way that supports both the person and the claim.

### Policy

Person-centered plans are reviewed on a stated schedule with the person served, and every service contact is documented within a defined timeframe.

### Procedure

1. The plan is reviewed with the person served no less than every 90 days, and additionally whenever a goal is achieved, the person's circumstances change materially, the person requests it, a crisis or critical incident occurs, or the person's other providers change the person's treatment.
2. The review documents, for each objective: progress in measurable terms; the person's own view of their progress in their own words; whether the objective is continued, revised, achieved, or discontinued; and the reason. An objective carried forward unchanged for two consecutive reviews requires a written explanation of why the approach has not changed.
3. The person signs and dates each review.
4. Every service contact is documented in a progress note completed no later than 24 hours after the contact, by the person who delivered the service, and signed with their name, credential, and the date signed.
5. Each progress note records: the date; the start and stop time; the location and the setting; who was present; the specific goal and objective addressed; what the peer support specialist actually did, described concretely; how the person responded, in the person's own words where possible; progress toward the objective; and the plan for next contact.
6. Notes are individualized. Copying a previous note, copying another person's note, or repeating the same intervention and response text across contacts is prohibited and is a documentation integrity issue under the Compliance policy.
7. A late entry is labeled as such with the date of the service and the date of the entry. A correction is made by a single line-through with initials and date, or by the amendment function in the electronic health record. Records are never erased, overwritten, or back-dated.
8. Documentation timeliness and quality are monitored under Quality Record Review and reported quarterly.

### Evidence the Surveyor Will Ask For

- Reviews completed on schedule, signed by the person served
- Notes with start and stop times and a goal reference
- Absence of copied or cloned narrative across notes and across persons served
- The timeliness data from record review

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_



## Section 2.D — Transition/Discharge

**What this area is about:** Transition planning begins at admission, not at the end. There is a written transition/discharge summary for every person who leaves, including unplanned exits, and a documented attempt at follow-up after discharge with the results recorded and analyzed.

**Questions a surveyor asks in this area:**

- Show me discharge summaries for the last five people who left, including the ones who just stopped coming.
- What does your follow-up data tell you?

**Who gets interviewed:** Program Director, Direct staff

### 2D-01 — Transition Planning

| Policy # | Owner                            | Review   | Standard area            |
|----------|----------------------------------|----------|--------------------------|
| 2D-01    | [FILL IN: Program Director name] | Annually | 2.D Transition/Discharge |

#### Purpose

To plan for the end of services from the day services begin, so no one falls off a cliff.

#### Policy

Transition planning begins at admission, is part of the person-centered plan, and is revisited at every plan review.

#### Procedure

1. At admission the person is asked what they want their life to look like when they no longer need this service, and that answer is recorded in their own words as the transition goal on the plan.
2. At each review, the plan records the anticipated transition timeframe, the criteria that will indicate readiness, the supports that will need to be in place, and what the person will do if they need help again.
3. Transition planning explicitly builds natural and community supports that will remain after paid service ends: mutual aid and recovery community connections, family and friends, faith community, employer, primary care, peer-run organizations. Progress in building them is documented.
4. Where a transition is expected, the receiving supports are contacted with the person's consent, and a warm hand-off is arranged - a direct introduction rather than a phone number on a piece of paper - and documented.
5. A person may choose to end services at any time. That choice is honored, documented, and does not prevent return.
6. An unplanned exit - the person stops attending, is incarcerated, moves, is hospitalized, or is lost to contact - triggers documented outreach: at least three attempts by at least two different methods over fourteen days, including a letter, before the record is closed.
7. Where Example Peer Recovery Services, Inc. initiates a discharge, the person is given the reason in writing, notice, the right to grieve the decision, and at least two alternative referral options with a warm hand-off offered.

#### Evidence the Surveyor Will Ask For

- A transition goal on the plan from admission, in the person's words
- Documented outreach attempts before an unplanned exit was closed
- A documented warm hand-off
- Written notice and grievance rights for any agency-initiated discharge

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## 2D-02 — Discharge Summary and Follow-Up

| Policy # | Owner                            | Review   | Standard area            |
|----------|----------------------------------|----------|--------------------------|
| 2D-02    | [FILL IN: Program Director name] | Annually | 2.D Transition/Discharge |

### Purpose

To close every record properly and to find out what happened to people after they left.

### Policy

A written transition or discharge summary is completed for every person who leaves Community Integration, for any reason, including unplanned exits, and follow-up contact is attempted after discharge.

### Procedure

1. The summary is completed within 10 days of the last service contact or of the decision to close, whichever is later, and is signed by the peer support specialist and the supervisor.
2. The summary records: the dates of service; the reason for discharge, including "unplanned - lost to contact" where that is what happened; the presenting needs at admission; the goals and the status of each at exit; the services delivered; the person's status at exit in their own words where obtainable; the supports and referrals in place at exit; any unmet need; medication and health follow-up arrangements where relevant; the crisis plan given to the person; and the conditions for return to service.
3. The person served is offered a copy and, where they participated, signs the summary. Where they did not participate, that is documented.
4. Follow-up contact is attempted at 30 days after discharge, with the person's prior consent, to ask how they are doing, whether the referrals connected, and whether they need anything. Attempts and outcomes are logged on the Follow-Up Contact Log.
5. Follow-up data - contact rate, status at follow-up, whether referrals connected, readmission - is aggregated at least annually and reported in the performance analysis. Follow-up outcomes are one of the effectiveness measures under the Performance Measurement Plan.
6. Records of discharged persons are retained per the Records Retention Schedule.

### Evidence the Surveyor Will Ask For

- A discharge summary for every closed record in the sample, including unplanned exits
- Goal status at exit, not just a narrative
- The Follow-Up Contact Log with results
- Follow-up data used in the annual analysis

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## Section 2.E — Medication Use

**What this area is about:** If the program does not prescribe, dispense, administer, or store medication, that must be stated in writing, and staff must know their limits and what to do about medication questions and adverse reactions. If it does any of those things, a full medication management policy set is required.

**Questions a surveyor asks in this area:**

- Do you touch medication in any way?
- Ask a peer specialist: what do you do if someone tells you they stopped their medication?

**Who gets interviewed:** Program Director, Direct staff

### 2E-01 — Medication

| Policy # | Owner                            | Review   | Standard area      |
|----------|----------------------------------|----------|--------------------|
| 2E-01    | [FILL IN: Program Director name] | Annually | 2.E Medication Use |

#### Purpose

To state exactly what Example Peer Recovery Services, Inc. does and does not do regarding medication, and what staff must do when medication comes up.

#### Policy

Example Peer Recovery Services, Inc. does not prescribe, dispense, administer, pre-pour, store, transport, or dispose of medication, and its personnel do not advise any person to start, stop, change, or skip a medication. Peer support personnel support a person's own self-management and their own conversation with their prescriber, and nothing more.

#### Procedure

1. Peer support specialists do not prescribe, dispense, administer, pre-pour, hand, store, transport, or dispose of medication, and do not advise a person to start, stop, change, or skip any medication. This limit is covered at orientation and is on the Competency Checklist.
2. Peer support specialists may, within their role: talk with a person about their own experience of medication as one part of their story; help a person write down questions for their prescriber; support a person to attend an appointment; help a person set up their own reminder system; and help a person self-advocate about side effects. All of this is education and support, not advice.
3. Example Peer Recovery Services, Inc. is neutral as to recovery pathway. Medication for addiction treatment, psychiatric medication, and medication-free pathways are all respected. No person is pressured, and no service is conditioned on a medication choice. Any staff statement discouraging prescribed medication is a Code of Ethics violation.
4. Where a person reports a missed dose, a side effect, or that they have stopped a medication, the staff member: supports the person to contact their prescriber the same day; offers to help make that call; documents what the person reported and what was done; and notifies the supervisor.
5. Where a person shows signs of a medical emergency, including a suspected overdose or a severe adverse reaction, the staff member calls emergency services immediately, administers naloxone if trained and indicated, stays with the person until help arrives, and files a Critical Incident Report.
6. If a staff member observes medication left unsecured in a home where a child or vulnerable adult is present, they raise it with the person served, document it, and notify the supervisor, who determines whether a report is required.
7. Staff never accept medication from a person served for any reason, including disposal.

#### Evidence the Surveyor Will Ask For

- The written statement of what the program does not do with medication
- Orientation and competency records covering it
- A documented instance of a staff member supporting a person to contact a prescriber
- Staff able to say, in interview, exactly where their limit is

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## Section 2.F — Nonviolent Practices

**What this area is about:** A written commitment to nonviolent practice: de-escalation training for staff, what is prohibited, what to do in a behavioral crisis, when to call for help, and a debriefing with the person served and staff after any crisis event.

**Questions a surveyor asks in this area:**

- What restraint or seclusion do you use? (correct answer for most peer programs: none, and it is prohibited in writing)
- Ask staff: what do you do if someone escalates in the car?
- Show me a debriefing after your last crisis event.

**Who gets interviewed:** Direct staff, Supervisor, Persons served

### 2F-01 — Nonviolent Practices

| Policy # | Owner                            | Review   | Standard area            |
|----------|----------------------------------|----------|--------------------------|
| 2F-01    | [FILL IN: Program Director name] | Annually | 2.F Nonviolent Practices |

#### Purpose

To commit in writing to nonviolent, trauma-informed practice, and to state what is prohibited.

#### Policy

Example Peer Recovery Services, Inc. prohibits restraint, seclusion, and every form of isolation, coercion, and physical intervention used to control a person's movement, in all of its programs, without exception. Example Peer Recovery Services, Inc. uses prevention, relationship, choice, and de-escalation, and never uses coercion.

#### Procedure

1. Prohibited at all times: any physical restraint; any mechanical restraint; any chemical restraint; seclusion; any form of isolation used as a consequence; withholding food, water, sleep, visits, phone contact, or personal property as a consequence; verbal abuse, threats, ridicule, shouting, and humiliation; and any physical contact used to control a person's movement, except the minimum contact required to remove oneself from immediate danger or to prevent imminent serious harm while emergency services are being summoned.
2. Prevention comes first. Staff learn each person's triggers, warning signs, and preferred calming strategies, and record them in the person's Personal Safety and Crisis Plan, written with the person.
3. All direct service staff complete de-escalation and trauma-informed care training at orientation and annually, documented on the De-escalation Training Log, and are assessed as competent.
4. In a behavioral crisis, staff: keep themselves and others safe; give the person space and time; speak calmly and offer choices; use the person's own stated calming strategies; involve someone the person trusts if the person wants that; and call the crisis line or emergency services when the risk of serious harm is imminent.
5. Staff leave the situation and call for help rather than intervene physically. Withdrawing is always the correct choice and is never a performance failure.
6. Any event in which emergency services were called, any physical contact occurred, or any person was injured or frightened is documented as a critical incident the same day.
7. The environment is reviewed for conditions that provoke escalation - crowding, noise, waiting, lack of privacy, power dynamics - and findings feed the Health and Safety and Accessibility plans.

#### Evidence the Surveyor Will Ask For

- The written prohibition, signed by staff
- De-escalation training and competency records for every direct service employee
- Personal Safety and Crisis Plans in records, written with the person
- Staff able to describe what they would do if someone escalated in a vehicle

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## 2F-02 — Crisis Response and Debriefing

| Policy # | Owner                                       | Review   | Standard area            |
|----------|---------------------------------------------|----------|--------------------------|
| 2F-02    | [FILL IN: Supervisor of the peer workforce] | Annually | 2.F Nonviolent Practices |

### Purpose

To respond well in a crisis and to learn from it afterwards, for the person and for the staff.

### Policy

Every person served has a crisis plan written with them, and every crisis event is followed by a documented debriefing.

### Procedure

1. Within 30 days of admission, and reviewed at each plan review, the person and the peer support specialist complete the Personal Safety and Crisis Plan recording, in the person's own words: what wellness looks like for them; their early warning signs; their triggers; what helps and what makes it worse; who they want called and who they do not; their preferred crisis service and hospital; their medications and prescriber, if they choose to record them; supports for children, pets, housing, and employment during a crisis; and what they want to happen after.
2. The person keeps a copy. With the person's consent, a copy is shared with the supports they name.
3. Crisis contact numbers for Guilford, Forsyth, Alamance, including 988, the local mobile crisis team, and the emergency number, are given to every person at admission and posted.
4. After any crisis event - an emergency call, a hospitalization, a suicide attempt, an overdose, a police contact, or any event that frightened anyone present - a debriefing is offered to the person served within two business days and held with staff within five business days.
5. The debriefing with the person served asks: what happened from their point of view; what helped; what made it worse; what they want done differently next time; and what needs to change in their crisis plan and person-centered plan. The plan is updated accordingly.
6. The staff debriefing asks the same questions from the staff perspective and additionally addresses the emotional impact on the staff member, including on their own recovery, and what support they need. Support offered includes supervision, time, and referral to the employee assistance or peer support resource.
7. Both debriefings are documented on the Crisis Debriefing Form. Debriefing is not an investigation and is not disciplinary; where a performance concern arises, it is handled separately.
8. Crisis events and debriefing findings are aggregated quarterly for trends and reported under Performance Improvement.

### Evidence the Surveyor Will Ask For

- A crisis plan in every record, in the person's own words, signed
- Completed debriefing forms for both the person served and staff after an event
- Evidence a plan was updated as a result
- The quarterly aggregate of crisis events

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## Section 2.G — Records of the Persons Served

**What this area is about:** A written list of what belongs in a record, who may write in it, how quickly entries must be made, how corrections are made, how records are secured, how long they are kept, and how they are destroyed. Entries are legible, dated, and signed with credential.

**Questions a surveyor asks in this area:**

- What is your documentation timeframe and can you prove notes meet it?
- Where are paper records kept and who has the key?
- Show me how a correction is made.

**Who gets interviewed:** Program Director, Direct staff, Records custodian

### 2G-01 — Content and Maintenance of the Record of the Person Served

| Policy # | Owner                            | Review   | Standard area                     |
|----------|----------------------------------|----------|-----------------------------------|
| 2G-01    | [FILL IN: Program Director name] | Annually | 2.G Records of the Persons Served |

#### Purpose

To define exactly what belongs in a record, so a surveyor pulling any file finds a complete one.

#### Policy

Example Peer Recovery Services, Inc. maintains one record for each person served, containing the items on the Record Content Checklist, maintained in the electronic health record.

#### Procedure

1. Every record contains at minimum:
  - identifying information, date of birth, contact information, and emergency contact;
  - the referral and screening documentation;
  - the signed Consent for Services and any specialized consents;
  - the signed rights acknowledgment and Orientation of Person Served Checklist;
  - the financial disclosure and fee agreement where applicable;
  - all Releases of Information and the disclosure log;
  - the person-centered plan and all reviews, signed and dated by the person;
  - the Personal Safety and Crisis Plan;
  - progress notes for every service contact;
  - documentation of referrals made and their outcomes;
  - correspondence and documentation of coordination with other providers;
  - documentation of any incident involving the person, in factual summary;
  - guardianship or representative documentation where applicable;
  - the transition or discharge summary and follow-up documentation.
2. Every entry is legible, dated, and signed with the writer's full name and credential. Entries are made in the record of the correct person; an entry made in the wrong record is corrected by amendment and the error is reported to the supervisor.
3. Records are stored in the electronic health record. Any paper record or printed material is kept in a locked cabinet in a locked room and is filed the same business day.
4. Records are never removed from the premises except as required for service delivery, and then only the minimum necessary, transported in a closed opaque container, never left unattended in a vehicle, and returned the same day.
5. The Record Content Checklist is used to audit records under Quality Record Review.

#### Evidence the Surveyor Will Ask For

- Any record pulled at random containing every checklist item
- Entries signed with name and credential
- Locked storage for any paper record
- The record content audit results

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## 2G-02 — Records Retention, Access, and Destruction

| Policy # | Owner                      | Review   | Standard area                     |
|----------|----------------------------|----------|-----------------------------------|
| 2G-02    | [FILL IN: Privacy Officer] | Annually | 2.G Records of the Persons Served |

### Purpose

To keep records as long as required, no longer, and to destroy them safely.

### Policy

Example Peer Recovery Services, Inc. retains records for the period required by law, payer contract, and this policy, and destroys them securely and verifiably.

### Procedure

1. The Records Retention Schedule states, for each record type, the retention period and the trigger date. Retention periods are the longest of: state law, payer contract, federal requirement, and organizational need. Where a period is not specified elsewhere, records of persons served are retained for 7 years from the date of last service, and for a minor, for 7 years from the date the person reaches the age of majority.
2. Personnel records, financial records, incident records, governance minutes, and accreditation documentation are retained per the schedule; governance minutes are retained permanently.
3. A person served may request access to their own record. Access is provided within thirty calendar days. Where access is denied in part, the reason is given in writing along with the right to a review.
4. A person may request an amendment to their record. The request and the response are documented, and where an amendment is refused, the person's written statement of disagreement is retained with the record.
5. Records under any legal hold, audit, or investigation are not destroyed, regardless of schedule, until the hold is lifted in writing.
6. Destruction is by shredding for paper and by secure sanitization for electronic media. A Certificate of Destruction records the record types, the date range, the method, the date, and the person responsible, and is retained permanently.
7. If Example Peer Recovery Services, Inc. closes, ceases a program, or transfers ownership, records are handled per applicable law, persons served are notified where required, and the custodian and location are documented before closure.

### Evidence the Surveyor Will Ask For

- The written Records Retention Schedule
- A completed request for access and the response, within thirty days
- Certificates of Destruction
- The written plan for records in the event of closure

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_



## Section 2.H — Quality Record Review

**What this area is about:** Records are reviewed on a written schedule using a written tool, by someone qualified, looking at whether documentation supports the service and the plan. Results are aggregated, trends identified, and corrective action tracked to closure.

**Questions a surveyor asks in this area:**

- Show me your last four quarterly record reviews.
- What did you find and what did you do about it?
- Show me that the correction actually happened.

**Who gets interviewed:** QI Coordinator, Supervisor

### 2H-01 — Quality Record Review

| Policy # | Owner                                      | Review   | Standard area             |
|----------|--------------------------------------------|----------|---------------------------|
| 2H-01    | [FILL IN: Quality Improvement Coordinator] | Annually | 2.H Quality Record Review |

#### Purpose

To check the records systematically, before a payer or a surveyor does.

#### Policy

Example Peer Recovery Services, Inc. conducts documented quality reviews of records of persons served no less than quarterly, using a written tool, and tracks every finding to correction.

#### Procedure

1. Reviews are conducted quarterly. The sample is at minimum ten percent of active records or five records, whichever is greater, and includes at least one record per person delivering direct service, at least one open record, and at least one closed record.
2. Reviews are conducted by the [FILL IN: Quality Improvement Coordinator] or a qualified designee who did not write the documentation being reviewed.
3. The Quality Record Review Tool checks at minimum: presence of every Record Content Checklist item; consent and rights documentation signed and current; the person-centered plan present, signed by the person, with goals in the person's own words and measurable objectives; reviews completed on schedule; every note linked to a current goal; notes timely, individualized, and signed with credential; service delivered matches the service authorized and billed, including date, time, duration, location, and rendering provider; no cloned or duplicated narrative within or across records; crisis plan present; referrals and coordination documented; and discharge documentation where applicable.
4. Each finding is recorded with the record identifier, the deficiency, the severity, the responsible staff member, the required correction, and a due date, on the Corrective Action Tracker.
5. Corrections are verified as complete by the reviewer. A correction is not closed on a promise; the corrected documentation is re-examined.
6. Where a finding indicates a service was billed that documentation does not support, the Compliance Officer is notified and the repayment procedure in 1F-02 applies.
7. Results are aggregated in the Record Review Summary Report by reviewer, by staff member, and by deficiency type, with trends identified and a written conclusion.
8. Individual results are reviewed with each staff member in supervision. A pattern of the same finding for the same staff member after correction is addressed as a competency and performance matter.
9. Summary results are reported to leadership quarterly and included in the annual performance analysis.

#### Evidence the Surveyor Will Ask For

- Four quarters of completed review tools with named reviewers and dates
- The Corrective Action Tracker showing findings closed and verified
- The quarterly summary reports and the trend analysis
- Supervision notes showing individual results were discussed

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## Section 3.CI — Community Integration

**What this area is about:** The program helps people participate in community life as they choose - housing, work, education, benefits, relationships, transportation, recreation, health care, and civic life. It is built on the person's own goals, uses natural supports and real community settings rather than segregated ones, and measures actual community participation, not just service contacts.

**Questions a surveyor asks in this area:**

- Show me a person who moved into their own housing, got a job, or joined something in the community because of you.
- How do you measure community participation?
- Ask a person served: what do you want that we are helping you get?

**Who gets interviewed:** Persons served, Direct staff, Program Director

### 3CI-01 — Community Integration Program Philosophy and Scope

| Policy # | Owner                            | Review   | Standard area              |
|----------|----------------------------------|----------|----------------------------|
| 3CI-01   | [FILL IN: Program Director name] | Annually | 3.CI Community Integration |

#### Purpose

To define what community integration means at Example Peer Recovery Services, Inc. and to hold the program to it.

#### Policy

Community Integration exists to support each person to participate in community life as fully as that person chooses, in the same places, at the same times, and alongside the same people as everyone else in the community.

#### Procedure

1. The program supports participation in at minimum these life domains, according to the person's own goals: housing of their own choosing; employment and income; education and training; benefits and entitlements; physical and behavioral health care; family, friendship, and intimate relationships; parenting and family reunification; transportation and mobility; money management and financial capability; legal matters and clearing barriers such as fines, licence suspension, and criminal record relief; recreation, leisure, faith, and culture; civic life, including voting; and connection to recovery community organizations and mutual aid of the person's chosen pathway.
2. Services are delivered in integrated community settings - real workplaces, real apartments, real classrooms, real coffee shops, real meetings - not in program-only settings, unless the person prefers otherwise and that preference is documented.
3. The program uses the principle of the least intrusive support that achieves the person's goal, and actively works to transfer support from paid staff to natural and community supports over time.
4. Congregate or segregated activity is not the default. Where a group activity is offered, participation is voluntary, and an individual alternative is available.
5. The program does not perform tasks for a person that the person can do or can learn to do; it supports the person to do them.
6. Success is defined by the person's own stated goals and by actual community participation, not by attendance or by units delivered. This is reflected in the measures in the Performance Measurement Plan.
7. The program description, the person-centered plan template, and the progress note template all reflect these principles.

#### Evidence the Surveyor Will Ask For

- Records showing goals across multiple life domains, not just one
- Notes showing service delivered in real community settings
- A person served who obtained housing, employment, education, or a community connection
- Measures that count community participation, not just service units

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## 3CI-02 — Choice, Self-Determination, and Dignity of Risk

| Policy # | Owner                            | Review   | Standard area              |
|----------|----------------------------------|----------|----------------------------|
| 3CI-02   | [FILL IN: Program Director name] | Annually | 3.CI Community Integration |

### Purpose

To protect the right of people to run their own lives, including the right to make choices others would not make.

### Policy

Every person served directs their own services and their own life. Example Peer Recovery Services, Inc. supports informed choice, including choices that carry risk, and does not use services as leverage.

### Procedure

1. The person chooses their own goals, the order in which they pursue them, where and when services occur, who provides them, and who else is involved. Each of these choices is documented in the plan.
2. A person may decline any service, activity, referral, or appointment. Declining has no effect on their access to other services, does not result in discharge, and is documented without pejorative language such as "non-compliant" or "resistant."
3. Where a person's choice carries foreseeable risk, staff support informed decision-making: they explain the likely consequences in plain language, offer alternatives, and document the discussion and the person's decision. The person's decision is then respected. This is the dignity of risk.
4. Where a choice presents an imminent risk of serious harm to the person or another, staff follow the crisis and mandated reporting procedures. Short of that threshold, staff do not override the person's decision.
5. Staff do not condition support, attention, transportation, or access on the person's compliance with a staff preference, sobriety, medication adherence, meeting attendance, or any pathway choice.
6. Where a person has a legal guardian, staff support the person's expressed preferences to the greatest extent the guardianship permits, document any conflict between the person's stated wish and the guardian's decision, and inform the person of supported decision-making and of their right to seek restoration of rights.
7. Supported decision-making, rather than substitute decision-making, is the default approach in every interaction.

### Evidence the Surveyor Will Ask For

- Documented declines with no adverse consequence
- A documented informed-risk discussion where the person's choice was respected
- Records free of "non-compliant" language
- Persons served able to say, in interview, that they chose their own goals

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## 3CI-03 — Natural Supports and Community Connection

| Policy # | Owner                            | Review   | Standard area              |
|----------|----------------------------------|----------|----------------------------|
| 3CI-03   | [FILL IN: Program Director name] | Annually | 3.CI Community Integration |

### Purpose

Paid support ends. Relationships do not. This policy makes building them part of the job.

### Policy

Example Peer Recovery Services, Inc. works deliberately to expand each person's network of unpaid, natural, and community supports, and measures whether that network grew.

### Procedure

1. At admission and at each plan review, the peer support specialist completes a Natural Supports Map with the person, showing who is in their life now, who they have lost contact with and would like to reconnect with, and what kind of connection they want more of.
2. At least one goal or objective on every person-centered plan addresses expanding natural or community support, unless the person declines, in which case the decline is documented.
3. Staff support the person to make connections rather than making them on the person's behalf: rehearsing a conversation, going with them the first time, then stepping back.
4. Staff actively step back as connections take hold, and document the reduction in paid support as a positive outcome rather than a loss of service units.
5. The program maintains and shares information about community groups, mutual aid across multiple pathways, recovery community organizations, faith communities, volunteering, classes, and recreation in Guilford, Forsyth, Alamance.
6. Growth in natural supports is measured for each person - the number and type of unpaid supports at admission compared with at review and at discharge - and is reported in aggregate as an effectiveness measure.
7. Family and natural supports are involved with the person's consent, are offered education about recovery and about peer support, and are treated as partners, not as informants.

### Evidence the Surveyor Will Ask For

- A completed Natural Supports Map in records, updated at review
- A natural-support goal on plans
- The aggregate measure showing whether natural supports grew
- Notes showing staff stepping back as a connection took hold

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## 3CI-04 — Transportation and Community Outings

| Policy # | Owner                            | Review   | Standard area              |
|----------|----------------------------------|----------|----------------------------|
| 3CI-04   | [FILL IN: Program Director name] | Annually | 3.CI Community Integration |

### Purpose

To move people safely and to build their independent mobility.

### Policy

Example Peer Recovery Services, Inc. transports persons served only under the conditions set out in this policy. The goal of any transport is to build the person's own ability to get where they need to go.

### Procedure

1. The first-choice approach is teaching and accompanying: helping the person learn the bus route, apply for reduced fare, use a rideshare application, plan a trip, or restore their driving privilege. Direct transport is a means to a goal, not a service in itself, and is documented against a goal.
2. Any staff member who transports a person served must have: a valid driver's licence verified annually; a driving record check at hire and annually; personal automobile insurance meeting the minimum stated by Example Peer Recovery Services, Inc. where a personal vehicle is used, verified annually; and written approval from the [FILL IN: Program Director name].
3. Vehicles used to transport persons served are inspected on the Vehicle Inspection form at least monthly, carry a first aid kit, a fire extinguisher, an emergency contact card, and a seat belt for every occupant, and are maintained on the manufacturer's schedule with records kept.
4. The person served gives written consent to transportation before the first trip, including acknowledgment of the risks.
5. Seat belts are worn by all occupants. Children are secured in an appropriate restraint provided by the parent or guardian; staff do not transport a child without an appropriate restraint.
6. Staff do not transport a person who is intoxicated to the point of being unsafe, who is behaviorally escalated, or who refuses a seat belt. The alternative arranged is documented.
7. Staff do not use a mobile device while driving, do not make personal stops, do not transport anyone not authorized, and do not leave a person served alone in a vehicle.
8. If a person escalates while the vehicle is moving, the driver pulls over safely, does not attempt physical intervention, and calls for assistance.
9. Any accident, near miss, or incident during transport is reported as a critical incident the same day.
10. Community outings are planned with the person against a goal, with the destination, purpose, and any known risk documented in advance for group outings, along with the staff-to-participant ratio and the plan if someone separates from the group.

### Evidence the Surveyor Will Ask For

- Driver licence, driving record, and insurance verification, dated within twelve months
- Monthly vehicle inspection forms
- Signed transportation consents
- Notes showing transport tied to a goal and to building independent mobility

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## 3CI-05 — Advocacy, Benefits, and Removing Barriers

| Policy # | Owner                            | Review   | Standard area              |
|----------|----------------------------------|----------|----------------------------|
| 3CI-05   | [FILL IN: Program Director name] | Annually | 3.CI Community Integration |

### Purpose

Most barriers to community life are practical. This policy makes removing them a documented part of the service.

### Policy

Example Peer Recovery Services, Inc. supports each person to obtain the benefits, documents, and remedies they are entitled to, and teaches self-advocacy so the person can do it themselves next time.

### Procedure

1. At admission the peer support specialist completes the Benefits and Entitlements Checklist with the person, covering at minimum: identification documents, birth certificate, and social security card; Medicaid, Medicare, and other health coverage; SSI and SSDI; SNAP; housing assistance and waiting lists; utility assistance; veterans' benefits; unemployment; vocational rehabilitation; child support and custody matters; outstanding fines, warrants, and licence suspensions; criminal record expungement eligibility; and voter registration.
2. Each gap identified becomes an objective on the person-centered plan with a target date, unless the person declines.
3. Staff support the person to complete applications rather than completing them for the person, and accompany the person to appointments where the person wants that.
4. Staff teach self-advocacy explicitly: how to prepare for an appointment, what to ask, how to request something in writing, how to appeal a denial, and how to keep a copy of everything.
5. Where a benefit is denied, staff support the person to appeal within the deadline and document the appeal and its outcome.
6. Staff never sign a document on behalf of a person served, never become a representative payee for a person served, never hold a person's identification, benefits card, cash, or bank access, and never co-sign a lease or loan for a person served.
7. Systemic barriers encountered - an office that will not take applications, a landlord who will not rent to people with records, a bus route that does not reach the clinic - are logged and reported annually to leadership for community advocacy under the strategic plan.

### Evidence the Surveyor Will Ask For

- A completed Benefits and Entitlements Checklist in records
- Objectives on plans arising from identified gaps
- A documented appeal of a denied benefit
- The systemic barrier log and evidence it reached leadership

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## Section 3.PEER — Peer Workforce Requirements (cross-cutting)

**What this area is about:** Peer support is a WORKFORCE and a service model, not a separate CARF program section. Its requirements are enforced through 1.I (credential, competency, supervision), 2.A (program description and staffing), 2.C (person-centered planning done WITH the person), and the Section 3 program. This area exists so nothing peer-specific falls between the cracks: lived experience as a job qualification, the state peer credential, self-disclosure boundaries, mutuality, dual-relationship management in small recovery communities, peer staff wellness, and career ladder.

**Questions a surveyor asks in this area:**

- What makes someone qualified to be a peer specialist here?
- How do you supervise self-disclosure?
- What happens when a peer specialist knows a participant from their own recovery community?
- How do you protect the wellness of staff in recovery?

**Who gets interviewed:** Peer Support Specialists, Supervisor, Program Director

### 3P-01 — The Peer Support Service Model

| Policy # | Owner                            | Review   | Standard area                                      |
|----------|----------------------------------|----------|----------------------------------------------------|
| 3P-01    | [FILL IN: Program Director name] | Annually | 3.PEER Peer Workforce Requirements (cross-cutting) |

#### Purpose

To define what peer support is at Example Peer Recovery Services, Inc., so that everyone - staff, persons served, payers, and surveyors - can tell it apart from clinical service.

#### Policy

Peer support at Example Peer Recovery Services, Inc. is a non-clinical, mutual, voluntary, recovery-oriented relationship in which a person with lived experience of a mental health or substance use condition supports another person's own recovery.

#### Procedure

1. Peer support at Example Peer Recovery Services, Inc. is characterized by:
  - **Mutuality** - the relationship goes both ways, and power is shared rather than held by the worker.
  - **Lived experience used purposefully** - the peer specialist's own recovery is a working tool, disclosed strategically, never as the centre of the conversation.
  - **Self-determination** - the person served sets the direction. The peer specialist does not assess, diagnose, treat, or direct.
  - **Hope** - the peer specialist is living evidence that recovery happens.
  - **Recovery-pathway neutrality** - every pathway is respected, including medication-assisted recovery, twelve-step, medication-free, faith-based, harm reduction, and self-directed recovery.
  - **Trauma-informed practice** - safety, trustworthiness, choice, collaboration, and empowerment in every interaction.
  - **Community focus** - the work happens where the person lives their life.
2. Peer support is not: therapy, counselling, clinical assessment, diagnosis, case management directed by a clinician, sponsorship, friendship, transportation on demand, or a substitute for treatment. Staff say so plainly when asked.
3. Peer support specialists work within the scope defined by their job description, their NC Certified Peer Support Specialist (NCCPSS) credential, and their assessed competency, and refer beyond it.
4. Where a person served needs clinical service, the peer specialist supports the connection and does not attempt to provide it.
5. The distinction between peer support and clinical service is covered at orientation, is on the Competency Checklist, and is explained to every person served during consent.

#### Evidence the Surveyor Will Ask For

- The written model in the program description and the handbook
- Peer specialists able to explain, in interview, what they do and do not do
- Persons served able to describe the relationship as mutual and self-directed
- Notes that read as peer support, not as clinical documentation

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_



## 3P-02 — Self-Disclosure

| Policy # | Owner                                       | Review   | Standard area                                      |
|----------|---------------------------------------------|----------|----------------------------------------------------|
| 3P-02    | [FILL IN: Supervisor of the peer workforce] | Annually | 3.PEER Peer Workforce Requirements (cross-cutting) |

### Purpose

Self-disclosure is the core tool of peer support and its most common failure point. This policy makes it deliberate and supervised.

### Policy

Peer support specialists at Example Peer Recovery Services, Inc. disclose their own recovery experience purposefully, in the service of the person served, and review that disclosure in supervision.

### Procedure

1. Before disclosing, the peer specialist asks themselves the test in this policy: Whose need does this meet? What do I want the person to take from it? Is this the shortest version that does that? Am I about to give advice disguised as a story?
2. Disclosure is appropriate when it: conveys hope, normalizes an experience, demonstrates that a barrier can be overcome, opens a door the person can choose to walk through, or invites the person to say more.
3. Disclosure is inappropriate when it: centres the peer specialist; prescribes the peer specialist's pathway as the right one; includes graphic detail of use, self-harm, or trauma; discloses current, unresolved struggle in a way that shifts support onto the person served; is used to fill silence; or exceeds what the peer specialist has decided in advance to share.
4. Every peer specialist prepares, with their supervisor, a written personal disclosure plan stating what parts of their story they are prepared to share, what they will not share, and how they will respond when asked something outside it. It is reviewed at least annually.
5. Self-disclosure is a standing item in every supervision session. Recent disclosures are reviewed for purpose and effect, using the Self-Disclosure Supervision Worksheet.
6. A peer specialist who is currently in acute personal crisis discusses with their supervisor whether and how they will disclose, and what support they need, without fear of adverse consequence.
7. Disclosure of the peer specialist's experience is documented in the progress note only as an intervention - for example, "shared lived experience of navigating a housing appeal to support the person's confidence in appealing" - and never in personal detail.
8. New peer specialists have at least one disclosure directly observed by the supervisor within their first ninety days.

### Evidence the Surveyor Will Ask For

- Written disclosure plans in personnel files
- Supervision notes that show disclosure was actually reviewed
- Notes that describe disclosure as an intervention without personal detail
- The observed disclosure for a new employee

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

### 3P-03 — Boundaries and Dual Relationships

| Policy # | Owner                                       | Review   | Standard area                                      |
|----------|---------------------------------------------|----------|----------------------------------------------------|
| 3P-03    | [FILL IN: Supervisor of the peer workforce] | Annually | 3.PEER Peer Workforce Requirements (cross-cutting) |

#### Purpose

In a small recovery community, the peer specialist and the person served often already know each other. This policy manages that rather than pretending it away.

#### Policy

Peer support specialists at Example Peer Recovery Services, Inc. maintain clear, documented boundaries and disclose every dual relationship.

#### Procedure

1. Prohibited at all times: any romantic or sexual contact with a person served; lending, borrowing, or giving money; entering a business or financial arrangement; accepting a gift beyond nominal value; providing housing to or living with a person served; using a person served for personal benefit, including as a customer, employee, or source of a substance; using or being under the influence of alcohol or any non-prescribed substance during service or with a person served; and attending a mutual aid meeting in the role of the person's sponsor while also being their paid peer specialist.
2. A dual relationship exists whenever the peer specialist has any relationship with the person served other than the professional one - a shared mutual aid group, a shared treatment history, a shared friend group, family connection, neighbour, former co-worker, a prior sponsor or sponsee relationship, or an existing online connection.
3. On becoming aware of any dual relationship, the peer specialist completes the Dual Relationship Disclosure before or immediately upon assignment, and no later than the next business day, and gives it to the supervisor.
4. The supervisor documents a management plan on that form. Options include: reassignment to another peer specialist; a written boundary agreement covering what will happen when both are present in a shared setting; increased supervision frequency; or, where the risk cannot be managed, non-acceptance of the referral with an alternative offered to the person.
5. A shared mutual aid meeting is not by itself a bar to service. Where both attend, they agree in advance, in writing, that neither will discuss the service relationship in the meeting, and that the peer specialist's participation there is as a community member, not as staff. The peer specialist's own recovery participation is never restricted by their employment.
6. Peer specialists do not counsel, sponsor, or supervise a family member or an intimate partner.
7. Boundary questions are a standing supervision item. A peer specialist who is uncertain raises it before acting; raising it is never treated as a performance concern.
8. Boundary crossings, including a person served appearing at the peer specialist's home, contacting them personally, or offering a gift, are reported to the supervisor the same day and documented.

#### Evidence the Surveyor Will Ask For

- Dual Relationship Disclosures with supervisor management plans
- Supervision notes addressing boundaries
- Staff able to describe, in interview, what they do when they meet a participant at a meeting
- The boundary policy acknowledgment signed in every personnel file

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## 3P-04 — Peer Workforce Wellness

| Policy # | Owner                   | Review   | Standard area                                      |
|----------|-------------------------|----------|----------------------------------------------------|
| 3P-04    | Chief Executive Officer | Annually | 3.PEER Peer Workforce Requirements (cross-cutting) |

### Purpose

The peer workforce is staffed by people in recovery doing emotionally demanding work. Protecting their wellness is an organizational obligation, not a personal one.

### Policy

Example Peer Recovery Services, Inc. actively supports the wellness and continued recovery of its peer workforce and does not penalize a staff member for attending to their own recovery.

### Procedure

1. Every peer support specialist develops a written Peer Wellness Plan with their supervisor within thirty days of hire, recording their own supports, their warning signs, what they need from the workplace when stressed, and what they want their supervisor to do if signs appear. It is reviewed at least annually and is kept in the confidential file, not the general personnel file.
2. Time to attend the staff member's own recovery supports - meetings, therapy, medical appointments - is accommodated through scheduling flexibility wherever operationally possible.
3. Caseload size, the volume of high-acuity work, and exposure to crisis and loss are monitored by the supervisor and adjusted when a staff member's wellness indicates it. Caseload is capped at the maximum in the staffing plan.
4. After a critical incident, a death of a person served, or any event with significant emotional impact, the supervisor offers a debriefing and additional support within five business days. Attendance is voluntary.
5. Staff may request a change in assignment, additional supervision, or time off for wellness reasons, and such a request is not recorded as a performance concern.
6. A staff member who experiences a return to use or a mental health crisis is supported. Example Peer Recovery Services, Inc. follows applicable employment law and any legal or credentialing reporting obligation, keeps the matter confidential to those with a need to know, offers a leave and a return-to-work plan, and does not terminate solely because of a health condition. Where the person's own safety or that of persons served requires it, duties are adjusted while support is arranged. Return to service requires clearance consistent with the credentialing body's rules.
7. Peer specialists have access to peer-to-peer consultation and to an employee assistance or equivalent resource, and the contact information is posted.
8. Staff turnover, and its stated reasons, is reviewed annually as part of the workforce analysis, with wellness explicitly considered.

### Evidence the Surveyor Will Ask For

- Peer Wellness Plans on file, in the confidential file
- Evidence of scheduling accommodation for recovery supports
- A documented post-incident support offer
- The turnover analysis addressing wellness

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_

## 3P-05 — Peer Specialist Credential, Scope, and Career Ladder

| Policy # | Owner                   | Review   | Standard area                                      |
|----------|-------------------------|----------|----------------------------------------------------|
| 3P-05    | Chief Executive Officer | Annually | 3.PEER Peer Workforce Requirements (cross-cutting) |

### Purpose

To define who may call themselves a peer support specialist at Example Peer Recovery Services, Inc., what they may do, and where they can go from here.

### Policy

Every peer support position at Example Peer Recovery Services, Inc. requires lived experience, the applicable state credential, and demonstrated competency, and has a defined scope and a path for advancement.

### Procedure

1. The minimum qualification for a peer support specialist at Example Peer Recovery Services, Inc. is: self-identified lived experience of a mental health condition, a substance use condition, or both, and of recovery from it; the NC Certified Peer Support Specialist (NCCPSS) credential, or eligibility for it with attainment within the timeframe stated in the job description; and any additional requirement of the payer.
2. A candidate's recovery status is a qualification. It is not documented in clinical detail in the personnel file, is not disclosed to persons served without the staff member's consent, and is never used as a basis for adverse action except as provided in the Peer Workforce Wellness policy.
3. Where a credential must be attained after hire, the person's duties are limited to those permitted for an uncredentialed worker under the payer's rules, and the limitation and the target date are documented. Service is not billed as peer support until the credential is verified.
4. Credentials, including continuing education requirements, are tracked to expiry by the Chief Executive Officer. Example Peer Recovery Services, Inc. supports continuing education through paid time and, where budgeted, fees.
5. Scope of practice for a peer support specialist at Example Peer Recovery Services, Inc. includes: engagement and relationship; sharing lived experience purposefully; supporting the person to set and pursue their own goals; recovery and wellness planning; skill building for community living; navigating systems and benefits; accompaniment; connection to natural and community supports; advocacy and teaching self-advocacy; crisis recognition and warm hand-off; and documentation.
6. Scope excludes: assessment, diagnosis, treatment, therapy, clinical supervision, medication handling or advice, legal or financial advice, and any service requiring a licence the person does not hold.
7. The career ladder defines the steps from Peer Support Specialist, to Senior Peer Support Specialist, to Peer Supervisor, and states the qualification, the added responsibility, and the compensation band for each. It is published to staff.
8. Peer staff are represented in organizational decision-making, including quality improvement and policy revision, and that participation is documented.

### Evidence the Surveyor Will Ask For

- Verified credentials for every peer support position, with expiry tracked
- Job descriptions stating lived experience as a qualification
- The published career ladder
- Evidence peer staff participated in a policy or QI decision

Approved: \_\_\_\_\_ Date: \_\_\_\_\_ Next review: \_\_\_\_\_